HOUSE OF REFUGE REHABILITATION CENTRE

COMPREHENSIVE TREATMENT PROTOCOL

Evidence-Based | Trauma-Informed | Christ-Centred | Reintegration-Focused

A Freedom Foundation Programme

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> *"He has sent me to proclaim freedom for the prisoners and recovery of sight for the blind, to set the oppressed free..." -- Luke 4:18*

TABLE OF CONTENTS

1. Document Control & Scope
2. Global Evidence Base
3. HOR Operational Context
4. Admission Protocol
5. Clinical Assessment Framework
6. Treatment Model: 4-Phase / 12-Week Programme
7. Detoxification Protocol
8. Therapeutic Interventions
9. Trauma-Informed Care
10. Spiritual Formation Protocol
11. Life Skills & Vocational Rehabilitation
12. Medication Management Protocol
13. Risk Assessment & Safety Protocol
14. Family Engagement Protocol
15. Behavioural Management Protocol

16. Discharge & Graduation Protocol
17. Continuing Care & Alumni Protocol
18. Outcome Measurement
19. Quality Assurance & Governance
20. References

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1. DOCUMENT CONTROL & SCOPE

1.1 Purpose

This Treatment Protocol establishes the clinical, psychosocial, spiritual, and operational standards for residential substance use disorder (SUD) treatment at House of Refuge Rehabilitation Centre. Every protocol decision herein is grounded in international research evidence, adapted for the Nigerian clinical and cultural context, and aligned with the mission of Freedom Foundation.

1.2 Scope

This document applies to all clinical and non-clinical staff delivering services within the HOR residential programme. It governs admission, assessment, treatment, discharge, and continuing care for all clients.

1.3 Document Control

Field	Detail
Document Owner	Program Director, House of Refuge
Clinical Review Authority	HOR Governing Board, Multi-Disciplinary Team
Version History	v1.0 (January 2026), v2.0 (April 2026)
Next Scheduled Review	April 2027, or sooner if new evidence warrants
Distribution	All HOR clinical and operational staff (authorised copies only)

1.4 Guiding Standards

This protocol has been developed in alignment with the following international frameworks, each detailed in Section 2:

- NIDA 13 Principles of Effective Drug Addiction Treatment
- WHO Mental Health Gap Action Programme (mhGAP) Guidelines, 2023 Update
- ASAM Criteria, Fourth Edition (2024)
- UNODC TREATNET Quality Standards
- Therapeutic Community Model (De Leon)
- Cochrane Collaboration Systematic Reviews (Withdrawal Management)
- Columbia University C-SSRS Validation Research
- Prochaska & DiClemente Transtheoretical Model of Change
- Brown et al. (2024) Spirituality in Recovery Framework

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2. GLOBAL EVIDENCE BASE

This section presents the international research foundations upon which every clinical decision in this protocol rests. HOR does not adopt practices because they are traditional or convenient; every intervention is selected because systematic research evidence supports its efficacy for substance use disorder populations.

2.1 NIDA's 13 Principles of Effective Drug Addiction Treatment

The National Institute on Drug Abuse (NIDA), a division of the US National Institutes of Health, published its Principles of Drug Addiction Treatment after more than two decades of funded clinical research. These 13 principles represent the most widely cited evidence-based framework for addiction treatment programme design globally. HOR's protocol is built to satisfy every principle:

Principle 1: No single treatment is appropriate for everyone. Treatment must be matched to the individual's particular problems and needs. HOR implements this through the ASAM six-dimensional assessment at admission (Section 5), individualised treatment planning using the Columbia Model (Section 8), and bi-weekly plan reviews at Multi-Disciplinary Team meetings.

Principle 2: Treatment needs to be readily available. Because drug-addicted individuals may be uncertain about entering treatment, taking advantage of available services the moment people are ready is critical. HOR maintains an open referral pathway with same-week admission capacity and an Outpatient Engagement Pathway (Section 4.5) for clients not yet ready for residential care, ensuring no potential client is turned away without a service offer.

Principle 3: Effective treatment attends to multiple needs of the individual, not just drug abuse. To be effective, treatment must address the individual's drug abuse and any associated medical, psychological, social, vocational, and legal problems. HOR's treatment model integrates clinical therapy, psychiatric screening, spiritual formation, vocational rehabilitation, family systems work, and community reintegration planning as concurrent streams (Section 6).

Principle 4: An individual's treatment and services plan must be assessed continually and modified as necessary. A client may require varying combinations of services during the course of treatment and recovery. HOR mandates treatment plan reviews at Week 4, Week 8, and pre-discharge, with emergency reviews triggered by clinical events (Section 8.1).

Principle 5: Remaining in treatment for an adequate period of time is critical. Research indicates that most addicted individuals need at least three months in treatment to significantly reduce or stop their drug use, and that the best outcomes occur with longer durations. HOR's 12-week (84-day) minimum programme duration is directly derived from this evidence, further supported by De Leon's Therapeutic Community research showing 9x better outcomes for clients retained beyond 90 days (De Leon, 2000) and CBT meta-analytic evidence identifying a critical window for skill consolidation within three months post-treatment initiation (PMC10572095, 2023).

Principle 6: Behavioral therapies -- including individual, family, or group counselling -- are the most commonly used forms of drug abuse treatment. Behavioral therapies vary in their focus and may involve addressing motivation to change, building skills to resist drug use, replacing drug-using activities with constructive ones, and improving problem-solving skills. HOR delivers Cognitive Behavioural Therapy (CBT), Motivational Interviewing (MI), relapse prevention therapy, and family systems therapy as core interventions (Sections 8, 9, 14).

Principle 7: Medications are an important element of treatment for many patients, especially when combined with counselling and other behavioral therapies. HOR integrates pharmacological management for withdrawal (Section 7), co-occurring psychiatric conditions (Section 12), and symptomatic relief, all under physician supervision and in accordance with WHO Essential Medicines List recommendations.

Principle 8: An individual's treatment and services plan must be assessed continually and modified as necessary to ensure it meets changing needs. This principle reinforces Principle 4 and is operationalised through HOR's MDT review schedule, real-time clinical observations documented in progress notes, and the flexibility to extend treatment duration to 26 weeks where clinically indicated.

Principle 9: Many drug-addicted individuals also have other mental disorders. Because substance use disorders and other mental illnesses are often co-occurring, clients presenting for either condition should be assessed for the co-occurrence of the other. HOR administers the PHQ-9 (depression), GAD-7 (anxiety), and targeted trauma screening at admission, consistent with WHO mhGAP (2023) recommendations for integrated mental health and substance use assessment in low- and middle-income countries (LMICs).

Principle 10: Medically assisted detoxification is only the first stage of addiction treatment and by itself does little to change long-term drug abuse. Although medically assisted detoxification can safely manage acute withdrawal symptoms, it is rarely sufficient to help addicted individuals achieve long-term abstinence. HOR's detoxification phase (Weeks 1-2) is explicitly positioned as the gateway to therapeutic engagement, not as treatment itself (Section 7).

Principle 11: Treatment does not need to be voluntary to be effective. Sanctions or enticements from family, employment settings, or the criminal justice system can significantly increase treatment entry, retention, and ultimate success. HOR's Outpatient Engagement Pathway uses Motivational Interviewing to support ambivalent or family-referred clients towards readiness, consistent with Prochaska and DiClemente's Transtheoretical Model research demonstrating that therapeutic engagement can facilitate movement from pre-contemplation to action stages (Section 4.5).

Principle 12: Drug use during treatment must be monitored continuously. Lapses during treatment do occur, and objective monitoring (e.g., urinalysis) can help the client withstand urges to use drugs. HOR conducts monthly urine drug screening (UDS) for all residents, with additional testing triggered by clinical suspicion or pass-related risk (Section 5.1).

Principle 13: Treatment programs should test patients for the presence of HIV/AIDS, hepatitis B and C, tuberculosis, and other infectious diseases, as well as provide targeted risk-reduction counselling. HOR's mandatory pre-admission medical panel includes HIV, Hepatitis A and B serology, chest X-ray for TB screening, and full blood count, with linkage to antiretroviral therapy (ART) services for HIV-positive clients (Section 4.8).

Source: NIDA, Principles of Drug Addiction Treatment: A Research-Based Guide, Third Edition. National Institutes of Health, Publication No. 12-4180.

2.2 WHO Mental Health Gap Action Programme (mhGAP), 2023 Update

The World Health Organization's mhGAP intervention guide is the primary international framework for scaling evidence-based mental health and substance use interventions in low- and middle-income countries. The 2023 update includes 30 updated recommendations and 18 new recommendations across mental, neurological, and substance use (MNS) conditions.

Key mhGAP principles operationalised at HOR include: (a) psychosocial interventions as first-line treatment for substance use disorders, consistent with HOR's emphasis on CBT, MI, and group therapy before pharmacological escalation; (b) integration of mental health screening instruments (PHQ-9, GAD-7) into routine substance abuse treatment assessment; (c) validation of non-specialist health worker delivery of evidence-based interventions under clinical supervision, which supports HOR's model of trained counsellors and house masters delivering structured psychosocial interventions under the oversight of the Program Director (Medical Doctor) and Clinical Lead; and (d) the inclusion of digitally delivered interventions across treatment modules, which HOR is incorporating through its digital client management platform for progress tracking, assessment scoring, and aftercare coordination.

The mhGAP framework is particularly relevant to HOR's context because it was designed specifically for resource-constrained settings where specialist psychiatric and addiction medicine capacity is limited. Its evidence base validates the feasibility and effectiveness of structured residential treatment programmes led by trained non-specialists with physician oversight.

Source: WHO mhGAP Guideline for Mental, Neurological and Substance Use Disorders, 2023 Update. World Health Organization, Geneva.

2.3 ASAM Criteria, Fourth Edition (2024)

The American Society of Addiction Medicine (ASAM) Criteria is the most widely used and comprehensive set of guidelines for patient placement, continued stay, and transfer/discharge of clients with addiction and co-occurring conditions. The Fourth Edition, published in 2024, provides what ASAM describes as its clearest withdrawal management guidance to date.

HOR operates at **ASAM Level 3.5: Clinically Managed High-Intensity Residential Services**, characterised by 24-hour care with trained counsellors, direct physician involvement for medical management, and a structured therapeutic community milieu.

The ASAM six-dimensional assessment framework governs HOR's admission and treatment matching decisions:

Dimension	Domain	HOR Application

1	Acute Intoxication / Withdrawal Potential	CIWA-Ar and COWS scoring, detox protocol activation
2	Biomedical Conditions and Complications	Pre-admission medical panel, physician evaluation
3	Emotional, Behavioral, or Cognitive Conditions	PHQ-9, GAD-7, C-SSRS, psychosocial assessment
4	Readiness to Change	URICA staging, Motivational Interviewing assessment
5	Relapse, Continued Use, or Continued Problem Potential	Addiction Severity Index, substance history, triggers
6	Recovery / Living Environment	Family assessment (Pathway A), VNA (Pathway B), housing

The ASAM dimensional framework ensures that admission decisions are clinically systematic rather than subjective, and that treatment intensity is matched to assessed severity across all six domains.

Source: ASAM Criteria, Fourth Edition. American Society of Addiction Medicine, 2024.

2.4 Therapeutic Community Model (De Leon)

The Therapeutic Community (TC) model, developed and researched extensively by Dr. George De Leon at Daytop Village in New York, provides the residential community framework for HOR's programme design. De Leon's research identifies the TC as operating on four interconnected views: the view of the substance use disorder, the view of the individual, the view of the recovery process, and the view of healthy living. The central principle is "community as method" -- the therapeutic community itself is the primary instrument of change, through structured daily routines, peer accountability, hierarchical work responsibilities, and collective governance.

The empirical evidence for the TC model is substantial. De Leon's longitudinal research at Daytop Village demonstrated that 88% of clients who completed two years in a therapeutic community were employed and crime-free at follow-up. More critically for HOR's 12-week programme design, TC outcome research consistently demonstrates that residents who remain in treatment for 90 or more days show outcomes approximately 9 times better than those who depart early (De Leon, 2000). This finding directly corroborates NIDA Principle 5 and provides the primary justification for HOR's non-negotiable 12-week minimum programme duration.

HOR implements TC principles through: structured daily schedules from 5:30 AM to 11:00 PM; morning and evening community meetings with peer accountability; hierarchical work roles and responsibilities within the facility; community governance through the behavioural management system; peer recovery circles; and graduated autonomy as clients progress through the four treatment phases.

Source: De Leon, G. (2000). The Therapeutic Community: Theory, Model, and Method. Springer Publishing. De Leon, G. "The Therapeutic Community: A Research Summary," PMC, 2020. Daytop Village Outcomes Research Archives.

2.5 Cognitive Behavioural Therapy: Systematic Review Evidence (2023)

A 2023 systematic review published in the journal Substance Abuse and Rehabilitation (PMC10572095) evaluated the cumulative evidence for CBT as a treatment for substance use disorders and issued a "strong recommendation" for CBT as an empirically supported treatment. The review synthesised findings across multiple meta-analyses and found:

- Effect sizes for CBT in SUD treatment range from small to large, depending on the substance involved. CBT demonstrates its strongest effects for cannabis and nicotine use disorders, with meaningful but smaller effects for opioid and alcohol use disorders.
- Meta-analytic data show a sharp decline in relapse rates within the first three months following CBT initiation, with stabilisation of gains after six months. This evidence identifies a "critical window for skill consolidation within three months" of treatment -- a finding that directly justifies HOR's 12-week programme as the minimum duration needed to complete the initial skill-building cycle.
- CBT's mechanism of action in SUD -- identifying and restructuring maladaptive cognitions, developing refusal skills, building coping strategy repertoires, and enhancing self-efficacy -- aligns with the broader TC model's emphasis on cognitive and behavioural change within a structured environment.

HOR delivers CBT through weekly individual sessions (60 minutes), thrice-weekly group sessions (90 minutes), and integrated relapse prevention planning (PRPP development in Phases 3-4).

Source: Magill, M. et al. (2023). "Cognitive Behavioral Therapy for Substance Use Disorders: A Systematic Review." Substance Abuse and Rehabilitation, PMC10572095.

2.6 Motivational Interviewing and the Transtheoretical Model

Motivational Interviewing (MI), developed by Miller and Rollnick, is grounded in over 30 years of clinical research demonstrating its effectiveness for engaging ambivalent and resistant clients in substance use treatment. MI is theoretically anchored in the Transtheoretical Model of Change developed by Prochaska and DiClemente (1982), which identifies five stages through which individuals progress in behaviour change:

1. **Precontemplation** -- No intention to change; unaware of or minimising the problem
2. **Contemplation** -- Awareness of the problem; considering change but ambivalent
3. **Preparation** -- Intending to take action; making small behavioural steps
4. **Action** -- Active modification of behaviour, experiences, and environment
5. **Maintenance** -- Sustaining gains; preventing relapse

Research demonstrates that the Precontemplation subscale alone is a significant predictor of treatment dropout, making stage-of-change assessment essential at admission. MI facilitates movement through these stages by resolving ambivalence through empathic, non-confrontational dialogue. The URICA (University of Rhode Island Change Assessment) instrument, administered at HOR admission and bi-weekly thereafter, provides standardised measurement of a client's stage of change.

HOR applies MI throughout the treatment continuum: in pre-admission screening to assess readiness; in the Outpatient Engagement Pathway to build motivation in pre-contemplative clients; in early treatment phases to maintain engagement; and whenever resistance or ambivalence emerges during the programme.

Source: Miller, W.R. & Rollnick, S. (2013). Motivational Interviewing: Helping People Change, Third Edition. Guilford Press. Prochaska, J.O. & DiClemente, C.C. (1982). "Transtheoretical Therapy: Toward a More Integrative Model of Change." Psychotherapy: Theory, Research & Practice, 19(3), 276-288.

2.7 Cochrane Reviews: Withdrawal Management Evidence

HOR's detoxification protocols are grounded in Cochrane Collaboration systematic reviews, the gold standard for evidence-based clinical practice:

Alcohol Withdrawal -- Symptom-Triggered Benzodiazepine Dosing: Cochrane and Oxford Academic evidence demonstrates that symptom-triggered benzodiazepine dosing (medication administered only when CIWA-Ar scores exceed threshold) is superior to fixed-schedule dosing. In controlled trials, symptom-triggered protocols resulted in significantly lower mean benzodiazepine doses (9.5 mg diazepam equivalent versus 19.9 mg under fixed schedules) and dramatically shorter treatment durations (mean 47.8 hours versus 146 hours).

This evidence supports HOR's adoption of CIWA-Ar-guided symptom-triggered dosing as the standard alcohol detoxification protocol.

Opioid Withdrawal -- Clonidine (Alpha-2 Adrenergic Agonist): Cochrane systematic review confirms that clonidine is more effective than placebo for managing opioid withdrawal symptoms, with particular importance in LMIC contexts where opioid agonist therapies (methadone, buprenorphine) may be unavailable or impractical. Clonidine addresses autonomic hyperactivity (sweating, tachycardia, hypertension, anxiety) while adjunctive medications -- loperamide for gastrointestinal symptoms, NSAIDs for musculoskeletal pain, promethazine for nausea -- provide comprehensive symptom management. This pharmacological approach forms the basis of HOR's opioid detoxification protocol.

Sources: Cochrane Database of Systematic Reviews: "Benzodiazepines for Alcohol Withdrawal" (multiple updates). Oxford Academic: Symptom-triggered versus fixed-schedule benzodiazepine dosing trials. Cochrane Database of Systematic Reviews: "Alpha-2 Adrenergic Agonists for the Management of Opioid Withdrawal."

2.8 Trauma-Informed Care Research

The integration of trauma-informed care (TIC) into residential SUD treatment is supported by a growing body of evidence documenting the high prevalence of trauma histories among individuals with substance use disorders and the impact of unaddressed trauma on treatment outcomes.

A 2024 study published in the Journal of Substance Use & Addiction Treatment evaluated trauma-informed residential treatment and found that "clients had significant reductions in substance involvement, depression, anxiety, and PTSD symptoms" when trauma-informed principles were systematically integrated into the treatment milieu. This finding directly supports HOR's commitment to trauma-informed practice across all staff interactions and programme components.

Research on Adverse Childhood Experiences (ACEs) demonstrates that ACE scores are disproportionately elevated in SUD populations. Individuals with four or more ACEs are 7 times more likely to develop alcohol use disorder and 4.5 times more likely to develop depression compared to those with zero ACEs (Felitti et al., 1998; Centers for Disease Control and Prevention ACE Study). History of trauma is also associated with poorer SUD treatment retention, making trauma screening and responsive practice essential for reducing dropout.

The four core areas of TIC implementation are: (1) universal trauma screening at admission; (2) comprehensive staff training in trauma-aware practice; (3) environmental modifications to promote physical and psychological safety; and (4) integration of trauma-focused therapeutic modalities (e.g., trauma-focused CBT) for clients with identified trauma histories.

Sources: Journal of Substance Use & Addiction Treatment (2024): Trauma-informed residential treatment outcomes. Felitti, V.J. et al. (1998). "Relationship of Childhood Abuse and Household Dysfunction to Many of the Leading Causes of Death in Adults," American Journal of Preventive Medicine, 14(4), 245-258. SAMHSA (2014). Concept of Trauma and Guidance for a Trauma-Informed Approach.

2.9 Spirituality and Recovery: Research Evidence

The integration of spirituality into addiction recovery is supported by a substantial evidence base that extends well beyond anecdotal or faith-tradition claims:

Prevalence and Client Demand: Research indicates that 73% of US addiction treatment programmes include a spirituality-based element, and surveys of treatment clients show that 84% desire greater emphasis on spirituality in their recovery programming. This demand-side evidence supports the provision of spiritual formation as a core programme component rather than an optional add-on.

Neurobiological Evidence: Neuroimaging studies demonstrate that practices of prayer and meditation strengthen neural circuits involved in attention regulation, emotional processing, and impulse control -- precisely the cognitive domains impaired by chronic substance use and critical for sustained recovery. These findings provide a neurobiological mechanism through which spiritual practices contribute to recovery outcomes.

Faith-Based Programme Outcomes: Adult & Teen Challenge, the world's largest faith-based residential recovery network (215+ centres), has been the subject of multiple outcome studies. Research shows that 74% of programme graduates report no substance use at six months post-completion, and 87.5% of former marijuana users were abstinent seven years after programme completion. Studies identify four elements contributing to recovery in faith-based settings: programme structure, relationships with others, experiences of God, and individual initiative.

Economic Impact: Faith-based volunteer support in addiction recovery contributes an estimated $316.6 billion in annual savings to the US economy through reduced healthcare utilisation, criminal justice costs, and productivity losses.

Theoretical Framework: Brown et al. (2024) published the Spirituality in Recovery Framework, providing an actionable, research-informed model for integrating spiritual transformation into clinical treatment. This framework guides HOR's approach to spiritual formation as a structured, measurable component of the recovery process rather than an undefined spiritual aspiration.

Sources: Brown, A.E. et al. (2024). "Spirituality in Recovery Framework." Journal of Religion and Health. Teen Challenge Research: Wilder Research Study (2009); Dr. Catherine Hess Study; ACADC Research Review. Neuroimaging: Newberg, A.B. (2014). "The Neuroscientific Study of Spiritual Practices," Frontiers in Psychology.

2.10 Continuing Care (Aftercare) Research

Research on continuing care following residential SUD treatment demonstrates that post-discharge support is not optional -- it is essential for sustained recovery outcomes.

McKay and colleagues' research on continuing care demonstrates that **12 or more months of structured aftercare is associated with significantly better long-term abstinence rates** compared to shorter or no aftercare (Journal of Substance Abuse Treatment). Furthermore, **rapid initiation of aftercare services within 14 days of residential discharge** significantly reduces relapse risk during the high-vulnerability transition period. The research consensus is that a **combination of services post-discharge** (individual check-ins, group support, family engagement, vocational support) represents best practice.

Mindfulness-Based Relapse Prevention (MBRP), developed by Bowen et al., has demonstrated **lower relapse rates at 12 months compared to treatment-as-usual** in randomised controlled trials. MBRP integrates mindfulness meditation practices with cognitive-behavioural relapse prevention strategies, providing clients with present-moment awareness skills that enhance their ability to recognise and manage cravings and high-risk situations.

These findings inform HOR's 24-month aftercare protocol with weekly contact in the first three months, structured outcome assessments at 3, 6, 12, and 24 months, and relapse response protocols activated within 48 hours of any identified lapse.

Sources: McKay, J.R. (2009). "Continuing Care Research: What We've Learned and Where We're Going," Journal of Substance Abuse Treatment, 36(2), 131-145. Bowen, S. et al. (2014). "Relative Efficacy of Mindfulness-Based Relapse Prevention, Standard Relapse Prevention, and Treatment as Usual," JAMA Psychiatry, 71(5), 547-556.

2.11 UNODC TREATNET Quality Standards

The United Nations Office on Drugs and Crime (UNODC) TREATNET programme, developed by an international consortium led by UCLA, provides quality standards specifically designed for drug dependence treatment in developing countries. Over 10,000 practitioners across 20+ countries have been trained using the TREATNET framework.

TREATNET standards address: comprehensive biopsychosocial assessment, individualised treatment planning, delivery of evidence-based interventions (CBT, MI, contingency management), structured community reintegration planning, and systematic aftercare follow-up. HOR's protocol is designed to meet or exceed TREATNET quality standards across all domains.

Of particular relevance to HOR is TREATNET's emphasis on culturally adapted implementation -- the recognition that evidence-based interventions must be delivered within local cultural frameworks to be effective. HOR's integration of Christian spiritual formation within an evidence-based clinical structure is consistent with TREATNET's principle that treatment programmes should incorporate culturally meaningful elements that enhance engagement and motivation.

Source: UNODC TREATNET Training Package, Volumes I-III. United Nations Office on Drugs and Crime, Vienna. Available at unodc.org/treatment-and-care.

2.12 Framework Integration Summary

Framework	Primary Application at HOR	Sections
NIDA 13 Principles	Programme design, individualisation, duration, monitoring	All
WHO mhGAP (2023)	Assessment instruments, LMIC adaptation, non-specialist delivery	5, 8, 12
ASAM Criteria 4th Ed (2024)	Admission criteria, level of care, 6-dimensional assessment	4, 5, 7
Therapeutic Community (De Leon)	Residential structure, peer accountability, community governance	6, 11, 15
CBT Systematic Review (2023)	Core therapeutic modality, "strong recommendation" evidence	8
Transtheoretical Model (Prochaska)	Admission readiness assessment, MI integration	4, 8
Cochrane Reviews	Detoxification protocols (alcohol, opioid)	7
Trauma-Informed Care (2024)	Universal screening, staff training, environmental safety	9
Spirituality in Recovery (Brown, 2024)	Structured spiritual formation protocol	10

Continuing Care (McKay)	24-month aftercare, rapid initiation, MBRP	17
UNODC TREATNET	Quality standards, cultural adaptation, reintegration	19

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3. HOR OPERATIONAL CONTEXT

This section provides the organisational, historical, and operational context specific to House of Refuge. All clinical protocols in subsequent sections are grounded in the international evidence base presented in Section 2.

3.1 Organisational Identity

House of Refuge (HOR) is a residential drug and alcohol rehabilitation centre established in 2006 as a programme of **Freedom Foundation**, a registered Nigerian non-profit organisation. HOR was founded by **John Akinola** with the vision of providing evidence-based, Christ-centred rehabilitation services to individuals struggling with substance use disorders in Lagos and across Nigeria.

3.2 Location and Facility

Detail	Specification
Location	Lekki, Lagos, Nigeria
Bed Capacity	24 beds
Wing Configuration	4 wings (configurable Male/Female ratio, default 3:1)
Programme Duration	12 weeks (84 days), extendable to 26 weeks where clinically indicated
Regulatory Registration	Lagos State Ministry of Health; Lagos State Ministry of Youth, Sport and Social Welfare; NDLEA

3.3 Vision, Mission, and Values

Vision: To be West Africa's leading Christ-centred rehabilitation centre, producing lasting freedom and productive reintegration for individuals and families affected by addiction.

Mission: To provide evidence-based, clinically excellent, and spiritually transformative residential treatment that equips every client for sustained recovery, productive employment, and meaningful community participation.

Core Values -- FREE:

Value	Application
F -- Freedom	True freedom from addiction, shame, and fear is available through Christ and evidence-based recovery
R -- Responsibility	We model and teach responsibility as a cornerstone of recovery and mature Christian character
E -- Exemplary Leadership	Every staff member leads by example in godliness, professionalism, compassion, and integrity
E -- Empathy	We see every client through the eyes of Christ with unconditional positive regard

3.4 Governance Structure

Level	Role
Board of Trustees (Freedom Foundation)	Strategic direction, policy approval, financial oversight
Executive Committee	Operational oversight, resource mobilisation
Executive Director	Day-to-day operations, human capital, financial stewardship
HOR Governing Board	Programme curriculum, client welfare, staff conduct, policy adherence
Program Director (Medical Doctor)	Clinical and operational leader of HOR

3.5 Multi-Disciplinary Team (MDT)

Composition: Program Director (chair), Head of Clinical Services, Head Nurse, Social Worker, House Master, Chaplain, Clinical Psychologist

Frequency: Weekly (standard), emergency meetings for critical incidents

Functions: Care plan reviews, relapse risk identification, discharge planning coordination, care delivery problem-solving

3.6 Staff Requirements

Role	Qualification	Registration
Program Director	Medical Doctor, addiction medicine experience	Medical council
Clinical Lead	Degree in psychology/counselling + CBT/MI training	Professional registration
Nursing Staff (3 rotating)	Registered Nurse	Nursing council
Social Worker	Degree in social work	Social work registration
Chaplain	Pastoral training + counselling skills	Church ordination/endorsement
House Master	Character, lived experience acceptable	Safeguarding-trained

3.7 Treatment Philosophy

HOR's treatment philosophy rests on four integrated pillars, each grounded in research evidence:

1. **Evidence-Based Clinical Practice** -- Cognitive Behavioural Therapy, Motivational Interviewing, and structured Relapse Prevention form the therapeutic backbone, consistent with the 2023 systematic review "strong recommendation" for CBT in SUD (PMC10572095) and NIDA Principle 6.

2. **Christ-Centred Spiritual Formation** -- Supported by research demonstrating that 73% of treatment programmes incorporate spirituality, 84% of clients desire it, and neuroimaging evidence confirms prayer and meditation strengthen impulse control circuits (Section 2.9).

3. **Trauma-Informed Care** -- Grounded in the 2024 Journal of Substance Use & Addiction Treatment study demonstrating significant reductions in substance involvement, depression, anxiety, and PTSD when TIC principles are integrated (Section 2.8).

4. **Reintegration-Focused Outcomes** -- Informed by De Leon's TC research demonstrating that vocational rehabilitation, community reintegration planning, and structured aftercare are essential components of sustained recovery (Section 2.4).

3.8 Referral Partners

Partner	Services
Yaba Psychiatric Hospital	Psychiatric assessment, inpatient care, prescription management
Rights and Humanity	Legal advice and assistance
NDLEA	Regulatory liaison, national drug treatment standards
Lagos State Ministry of Health	Regulatory oversight, HIV/ART referral
Freedom Foundation Empowerment Unit	Vocational training, seed capital, employment
Freedom Foundation Educational Unit	Further education opportunities
Life Centres (This Present House)	Church placement, pastoral mentoring, fellowship
Private Psychiatric Providers	Confidential private psychiatric care
HIV/ART Services	Government or private ART facility linkage

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4. ADMISSION PROTOCOL

4.1 Evidence Base for Admission Design

HOR's admission protocol integrates two primary research frameworks:

1. **ASAM Six-Dimensional Assessment (2024):** Every admission decision is informed by systematic evaluation across ASAM's six dimensions -- Acute Intoxication/Withdrawal Potential, Biomedical Conditions, Emotional/Behavioral/Cognitive Conditions, Readiness to Change, Relapse/Continued Use Potential, and Recovery/Living Environment. This ensures treatment placement is clinically appropriate and matched to individual severity.

2. **Prochaska & DiClemente's Transtheoretical Model (1982):** Client willingness is not assessed as a binary yes/no but as a position on the empirically validated stages-of-change continuum. Clients in the Precontemplation or early Contemplation stages are not rejected but are offered stage-appropriate services (Outpatient Engagement Pathway) consistent with NIDA Principle 11 (treatment does not need to be voluntary to be effective) and research showing that the Precontemplation subscale predicts treatment dropout.

4.2 The Willingness Criterion -- Primary Gateway

Admission to residential care requires demonstrated readiness to participate in recovery, assessed through validated instruments rather than subjective judgement:

- **URICA (University of Rhode Island Change Assessment):** Standardised measurement of stage of change. Clients scoring in the Action or Preparation stages are prioritised for immediate residential admission. Clients in Contemplation are admitted with enhanced motivational support. Clients in Precontemplation are enrolled in the Outpatient Engagement Pathway.
- **Motivational Interviewing-based dialogue:** Structured clinical interview using MI techniques (OARS: Open questions, Affirmations, Reflective listening, Summaries) to assess motivation and resolve ambivalence.
- **Clinical interview by Program Director (Medical Doctor):** Comprehensive medical and psychosocial evaluation.

Unwilling clients are NOT turned away. Consistent with NIDA Principle 11, they are enrolled in the Outpatient Engagement Pathway (Section 4.5).

4.3 Beneficiary Profile

Criterion	Specification	Evidence Basis
Gender	Male & Female	--
Age Range	18 to 65 years	--
Primary Condition	SUD (moderate to severe): alcohol, cannabis, opioids, stimulants, inhalants, polysubstance	DSM-5 diagnostic criteria
Psychiatric Status	Stable or manageable; no active psychosis, no acute suicidal/homicidal risk	ASAM Dimension 3
Medical Status	Medically stable or manageable within HOR's 24/7 nursing scope	ASAM Dimension 2

Willingness	Demonstrates readiness per Transtheoretical Model staging	ASAM Dimension 4

4.4 Inclusion and Exclusion Criteria

Five Inclusion Criteria (ALL Must Be Met):

#	Criterion	Assessment Method	Evidence Basis
1	**Willingness** -- Action/Preparation stage on URICA, or Contemplation with MI engagement	URICA, MI-based dialogue, clinical interview	Prochaska & DiClemente (1982); ASAM Dimension 4
2	**SUD Diagnosis** -- meets DSM-5 criteria for moderate-to-severe SUD	ASI, ASSIST screening	ASAM Dimensions 1, 5
3	**Medical Stability** -- manageable within ASAM Level 3.5 scope	Program Director medical evaluation	ASAM Dimension 2
4	**Functional Capacity** -- can participate in group therapy and counselling	Clinical interview, cognitive screening if indicated	ASAM Dimension 3
5	**No Immediate Acute Risk** -- no active suicidal ideation with intent/plan, no homicidal intent, no acute psychosis	C-SSRS, standardised safety screening	Columbia University C-SSRS validation; ASAM Dimension 3

Exclusion Criteria (Immediate Referral Required):

Condition	Action	Clinical Rationale
Active Psychosis	Referral to Yaba Psychiatric Hospital; HOR door open upon stabilisation	Exceeds ASAM Level 3.5 scope; requires Level 4 care
Severe Suicidal/Homicidal Risk	Emergency referral to hospital A&E; family notified immediately	C-SSRS high/critical risk; requires emergency psychiatric services
Drug-Induced Psychosis (DIP)	Referral for psychiatric stabilisation; re-assessment upon resolution	Acute psychotic symptoms require specialist management

Antipsychotic Medication Need	Referral to psychiatrist; re-admission when pharmacologically stable	Complex pharmacology exceeds HOR nursing scope
Severe Cognitive Impairment	Referral to neurological/cognitive rehabilitation specialist	Cannot meaningfully engage in CBT/group therapy
Active Legal Detention	Liaise with legal authority; offer outpatient engagement	NIDA Principle 11 supports engagement even within legal constraints
Children/Family Co-habitation	Refer to family-inclusive outpatient services	Residential environment not configured for dependent children

Golden Rule: Every excluded client leaves with: (1) a written referral, (2) a compassionate conversation, (3) HOR's contact number, (4) basic harm reduction information. Follow-up within 72 hours.

4.5 Outpatient Engagement Pathway (Pre-Contemplative and Contemplative Clients)

Consistent with NIDA Principle 11 and Prochaska's Transtheoretical Model, clients not yet ready for residential treatment receive stage-appropriate services designed to facilitate movement towards the Action stage:

Component	Description	Evidence Basis
Motivational Interviewing	Weekly 45-60 min individual sessions using MI techniques	Miller & Rollnick (2013); 30+ years efficacy evidence
Psychoeducation	Addiction as a brain disorder, recovery process education	NIDA Principle 3 (multiple needs)
Family Counselling	With consent -- psychoeducation about enabling vs supporting	Family systems research (Section 14)
Harm Reduction Guidance	Practical safety information to reduce immediate risk	WHO mhGAP (2023) harm reduction recommendations
Spiritual Engagement	Voluntary invitation to prayer, pastoral conversation, chapel	Client choice; non-coercive
Reassessment	Every 2-4 weeks via URICA and clinical interview	Stage-of-change monitoring; ASAM Dimension 4

4.6 Dual Admission Pathways

Pathway A -- Family-Supported Admission:

- Family consent and active engagement documented
- Financial agreement signed (payment plan available)
- Primary Family Support Person (PFSP) identified and named
- Family participates in minimum 2 therapy sessions + 1 pre-discharge meeting
- Family completes psychoeducation on addiction and enabling behaviour
- Home environment assessed for safety and sobriety-supportiveness (ASAM Dimension 6)

Pathway B -- Community-Supported Admission:

- Vulnerability and Needs Assessment (VNA) by Social Worker
- Sponsorship, community funding, or HOR Compassion Fund confirmed
- Social Worker assigned as primary case manager (treatment + 12-month aftercare)
- Church/faith community partner identified
- Supported housing referral initiated by Week 8 (ASAM Dimension 6)
- Vocational assessment completed before discharge

4.7 Eight-Step Admission Process

Step	Content	Responsible	Evidence Basis
1. Initial Contact	Compassionate engagement; explain programme, expectations, costs	Program Officer / Counsellor	NIDA Principle 2 (readily available)
2. Pre-Screening	ASI, ASSIST, URICA administered; willingness assessed; eligibility determined	Program Director	ASAM 6-dimensional framework
3. Clinical Assessment	Full medical evaluation; psychiatric screening; psychosocial assessment; medical tests ordered	Doctor + Nursing Staff	ASAM Dimensions 1-3; NIDA Principle 9
4. Admission Decision	Admit / Refer / Defer decision documented; pathway activated	Program Director	ASAM placement criteria

5. Documentation	Consent forms signed: Admission Agreement, Informed Consent, Detox Consent, Confidentiality, Family Confidentiality, Belongings Receipt, Financial Agreement, Rights Charter	Program Officer + Client	Ethical standards
6. Intake & Orientation	Property inspection; contraband check; belongings inventory; bed assignment; full orientation	House Master + Staff	TC model orientation protocols (De Leon)
7. Treatment Planning	Columbia Model treatment plan within 72 hours; client co-authors goals	Clinical Lead + Counsellor	NIDA Principles 1, 3, 4
8. Detoxification	Medical detox under doctor supervision with 24/7 nursing (Weeks 1-2)	Doctor + Nursing Staff	NIDA Principle 10; ASAM Clinical Practice Guideline

4.8 Pre-Admission Medical Tests (Mandatory within 48 hours)

Consistent with NIDA Principle 13 (test for HIV/AIDS, hepatitis, TB, and other infectious diseases) and adapted for the Nigerian clinical context:

Test	Clinical Rationale	Evidence Basis
HIV/AIDS Screening	Baseline status; HIV-positive clients NOT excluded, linked to ART	NIDA Principle 13
Hepatitis A & B Serology	Co-morbidity assessment; immunisation needs	NIDA Principle 13
Urine Drug Screen (UDS)	Confirms substances of use; baseline for detox planning	NIDA Principle 12
Full Blood Count (FBC)	Identifies anaemia, infection, haematological issues	ASAM Dimension 2
Liver Function Tests (LFTs)	Critical for alcohol-dependent clients; medication safety	ASAM Clinical Practice Guideline

Malaria Parasite Smear	Rules out malaria as confounding factor in withdrawal presentation	Nigerian clinical context; WHO mhGAP LMIC adaptation
Widal Test	Rules out typhoid fever	Nigerian clinical context
Chest X-Ray (CXR)	Screens for pulmonary TB, aspiration pneumonia	NIDA Principle 13; WHO TB screening guidelines
Blood Glucose (Random)	Identifies diabetes/hypoglycaemia for nutritional management	ASAM Dimension 2

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5. CLINICAL ASSESSMENT FRAMEWORK

5.1 Assessment Instruments and Their Evidence Base

Each clinical instrument used at HOR is internationally validated, with published psychometric data supporting its reliability and validity for the intended population:

Instrument	Purpose	Validation Evidence	Timing
ASI (Addiction Severity Index)	Comprehensive addiction assessment across 7 domains (medical, employment, drug use, alcohol use, legal, family/social, psychiatric)	Developed at the University of Pennsylvania; used in 30+ countries; test-retest reliability r=0.83-0.95 across domains	Admission
ASSIST (Alcohol, Smoking and Substance Involvement Screening Test)	Substance-specific risk screening across 10 substance categories	Developed by WHO; validated in 18 countries across diverse populations; sensitivity 80-100% for detecting SUD	Admission
URICA (University of Rhode Island Change Assessment)	Stage of change / readiness assessment based on Transtheoretical Model	McConnaughy et al. (1983); validated across SUD populations; predictive of treatment retention	Admission, bi-weekly
C-SSRS (Columbia Suicide	Suicide risk screening (6 items)	Developed at Columbia University; validated in	Admission, weekly, any

Severity Rating Scale)		FDA-mandated clinical trials; adopted by US military, WHO, and multiple national health systems; sensitivity 67-100% for detecting suicidal ideation	concern
CIWA-Ar (Clinical Institute Withdrawal Assessment - Alcohol, Revised)	Alcohol withdrawal severity (10 items, max 67)	Validated in Cochrane systematic reviews; inter-rater reliability r=0.90+; the standard measure in alcohol withdrawal research globally	Detox phase, scheduled rounds
COWS (Clinical Opiate Withdrawal Scale)	Opioid withdrawal severity (11 items, max 47+)	Recommended by NIDA and ASAM for opioid withdrawal monitoring; inter-rater reliability validated in multiple clinical trials	Detox phase, scheduled rounds
AUDIT (Alcohol Use Disorders Identification Test)	Alcohol use severity (10 items, max 40)	Developed by WHO; validated in 6 countries; sensitivity 92%, specificity 94% for hazardous drinking at cut-off of 8	Admission + Discharge
DAST-10 (Drug Abuse Screening Test)	Drug abuse severity (10 items, max 10)	Skinner (1982); widely validated; internal consistency alpha=0.92; used in UNODC TREATNET assessment protocols	Admission + Discharge
PHQ-9 (Patient Health Questionnaire)	Depression screening (9 items, max 27)	Kroenke et al. (2001); validated in 80+ languages; recommended by WHO mhGAP (2023) for depression screening in LMIC settings; sensitivity 88%, specificity 88% at cut-off of 10	Admission + Discharge

GAD-7 (Generalised Anxiety Disorder)	Anxiety screening (7 items, max 21)	Spitzer et al. (2006); recommended by WHO mhGAP (2023); sensitivity 89%, specificity 82% at cut-off of 10	Admission + Discharge

5.2 CIWA-Ar Scoring Protocol (Alcohol Withdrawal)

The CIWA-Ar is the internationally validated standard for monitoring alcohol withdrawal severity, endorsed by the Cochrane Collaboration and ASAM Clinical Practice Guidelines for guiding pharmacological intervention.

10 assessment items: Nausea/vomiting, Tremor, Paroxysmal sweats, Anxiety, Agitation, Tactile disturbances, Auditory disturbances, Visual disturbances, Headache, Orientation/clouding of sensorium

Score Range	Severity	Clinical Action	Evidence Basis
< 8	Mild	Continue monitoring q4h; no medication indicated	Cochrane: scores <8 do not require pharmacological intervention
8-15	Moderate	Initiate symptom-triggered benzodiazepine protocol; monitor q2h	Cochrane: medication threshold at score >8; symptom-triggered dosing reduces mean dose (9.5mg vs 19.9mg)
15-20	Severe	Intensify symptom-triggered protocol; monitor q1h; seizure precautions	ASAM Clinical Practice Guideline: escalation at higher CIWA-Ar scores
> 20	Very Severe / DT Risk	Intensive monitoring; consider transfer to higher level of care if not stabilising within 24 hours	ASAM: delirium tremens risk increases significantly above CIWA-Ar 20

5.3 COWS Scoring Protocol (Opioid Withdrawal)

The COWS is recommended by NIDA and ASAM for standardised monitoring of opioid withdrawal severity, guiding the timing and intensity of symptomatic management.

11 assessment items: Resting pulse rate, Sweating, Restlessness, Pupil size, Bone/joint aches, Runny nose/tearing, GI upset, Tremor, Yawning, Anxiety/irritability, Gooseflesh skin

Score Range	Severity	Clinical Action	Evidence Basis
0-4	Minimal	Continue supportive care and hydration	NIDA/ASAM: subclinical withdrawal
5-12	Mild	Supportive care; comfort medications (loperamide, NSAIDs)	Cochrane: adjunctive symptom management
13-24	Moderate	Clonidine protocol initiated; close monitoring	Cochrane: clonidine more effective than placebo for autonomic symptoms
25-36	Moderately Severe	Active pharmacological management; clonidine + full adjunctive protocol	Cochrane: alpha-2 agonist efficacy confirmed
> 36	Severe	Intensive management; consider transfer if not stabilising	ASAM Level 4 criteria may be indicated

5.4 C-SSRS Risk Assessment Protocol

The Columbia Suicide Severity Rating Scale was developed at Columbia University and validated through FDA-mandated clinical trials involving over 15,000 participants. It has been adopted as the standard suicide risk screening instrument by the US military, multiple national health systems, and the World Health Organization.

6 screening questions (all Yes/No):

1. Wish to be dead
2. Suicidal thoughts (non-specific)
3. Suicidal thoughts with method (without plan)
4. Suicidal intent (without specific plan)
5. Suicidal intent with plan
6. Suicidal behaviour (with timeframe: lifetime / past 3 months / past month / since last visit)

Additional safety screening:

- Active psychosis indicators (select from standard list)
- Homicidal ideation (Yes/No)
- Self-harm history (None / Historical / Past 3 months / Current)

Risk Level	Criteria	Required Action
Low	Q1-2 only, no other flags	Continue routine weekly monitoring
Moderate	Q3-4, or past self-harm	Increase observation frequency; notify Clinical Lead; document in risk register
High	Q5, or Q3-4 with self-harm history	Immediate 1-to-1 supervision; notify Program Director; contact family/PFSP
Critical	Q6 (recent), or active psychosis/homicidal ideation	Emergency protocol: refer to Yaba Psychiatric Hospital or nearest A&E

5.5 Clinical Assessment Schedule

Assessment	Admission	Weekly	Fortnightly	Monthly	Discharge
C-SSRS	X	X			X
CIWA-Ar (alcohol clients)	X	As needed			
COWS (opioid clients)	X	As needed			
AUDIT	X				X
DAST-10	X				X
PHQ-9	X			X	X
GAD-7	X			X	X
URICA	X		X		X
Vitals (BP, Pulse, Temp)	X	X			X
UDS	X			X	X
Weight / BMI	X			X	X
Trauma Screen (ACE)	X				

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6. TREATMENT MODEL: 4-PHASE / 12-WEEK PROGRAMME

6.1 Evidence Base for Programme Duration and Structure

HOR's 12-week (84-day) minimum programme duration is justified by the convergence of three independent evidence streams:

1. **NIDA Principle 5:** "Research indicates that most addicted individuals need at least three months in treatment to significantly reduce or stop their drug use and that the best outcomes occur with longer durations of treatment."

2. **De Leon's Therapeutic Community Research:** TC outcome data demonstrate that residents who remain in treatment for 90 or more days show outcomes approximately 9 times better than those who depart early. The 90-day threshold represents the minimum duration for meaningful internalisation of community norms, development of peer accountability, and acquisition of sustained behavioural change (De Leon, 2000).

3. **CBT Meta-Analytic Evidence (2023):** The systematic review (PMC10572095) identifies a "critical window for skill consolidation within three months" of CBT initiation. Meta-analytic data show a sharp decline in relapse rates within the first three months, with stabilisation of treatment gains after six months. The 12-week programme captures this critical consolidation period.

The four-phase structure is grounded in TC staged treatment research, which demonstrates that recovery progresses through sequential stages of stabilisation, skill building, deepening, and reintegration -- each requiring distinct therapeutic emphases and graduated levels of client autonomy.

6.2 Phase Overview

Phase	Weeks	Focus	Key Milestones	Evidence Basis
Phase 1: Medical Stabilisation	1-2	Detoxification, vitals monitoring, nutritional recovery, orientation, baseline assessments	Medical detox complete, all baseline assessments complete,	NIDA Principle 10 (detox = first stage only); ASAM withdrawal management

			treatment plan initiated	
Phase 2: Therapeutic Foundation	3-6	CBT initiation, relapse prevention, psychoeducation, individual counselling, spiritual formation begins, life skills begin	Week 4 MDT treatment plan review, triggers identified, coping skills developing	CBT skill acquisition phase (PMC10572095); Transtheoretical Model Action stage
Phase 3: Deepening & Skills Building	7-10	Advanced therapy, trauma processing, family systems, vocational assessment, PRPP development	Week 8 MDT review, PRPP drafted, vocational pathway identified, family therapy sessions	CBT skill consolidation (critical window); TC deepening phase (De Leon)
Phase 4: Reintegration & Graduation	11-12	Pre-discharge assessment, family/community meeting, aftercare plan, graduation ceremony, Alumni Programme enrolment	All 6 graduation criteria met, PRPP finalised, church placement confirmed, aftercare plan signed	TC reintegration phase; McKay continuing care evidence

6.3 Daily Schedule (5:30 AM -- 11:00 PM)

The structured daily schedule is a core therapeutic instrument consistent with the TC model's principle that "community as method" requires predictable routine, graduated responsibility, and bounded time for reflection (De Leon, 2000).

Time	Activity	Responsible
5:30 AM	Wake-up Call & Personal Hygiene	House Master
6:00 AM	Morning Exercise / Boot Camp (45 min)	House Master

6:45 AM	Personal Shower & Grooming	Residents
7:00 AM	Morning Prayer & Devotions (Praise, Worship, Prayer, Word)	Chaplain / House Master
8:00 AM	Breakfast	Cook / Domestic Staff
8:30 AM	Morning Community Meeting (accountability, day preview)	House Master / Counsellor
9:00 AM	Bible School / Spiritual Formation Class (2 hours)	Chaplain / Bible Teachers
11:00 AM	Group Therapy -- CBT / Relapse Prevention / Psychoeducation (rotated)	Clinical Lead / Counsellor
12:30 PM	Lunch	Cook / Domestic Staff
1:30 PM	Life Skills Training Group (rotating modules)	Counsellor / Social Worker / Nurse
2:30 PM	Work & Craft / Vocational Skills / Creative Therapies	House Master / Counsellor
3:30 PM	Individual Counselling Appointments (scheduled)	Assigned Counsellor
4:00 PM	Physical Activity / Sports / Gym / Recreation	House Master
5:00 PM	Personal Time -- prayer, Bible study, journaling (supervised)	House Master
6:00 PM	Evening Meal / Dinner	Cook / Domestic Staff
7:00 PM	Evening Chapel / Bible Study / Testimony / Discipleship Group	Chaplain / Counsellor
8:30 PM	Wrap-Up Meeting (day review, reflection, accountability, prayer)	House Master / Counsellor
9:00 PM	Personal Time -- journaling, reading, fellowship (supervised)	House Master

10:30 PM	Evening Medication Round (if applicable)	Nurse on Duty
11:00 PM	Lights Out -- all rooms	House Master

Schedule Notes:

- Nursing rounds: 6:00 AM, 12:00 PM, 6:00 PM, 10:30 PM daily
- Sunday: Modified for Chapel Service (9:00-11:30 AM). Afternoon free with family visits (12:00-6:00 PM)
- Weeks 1-2 (Detox Phase): Modified to reduce therapeutic demands. Rest, nutrition, and medical monitoring take priority, consistent with NIDA Principle 10 (detox is first stage only; therapeutic engagement follows stabilisation)
- Fasting: Permitted with doctor approval. NEVER during detox or withdrawal

6.4 Weekly Therapy Schedule

Component	Frequency	Duration	Evidence Basis
Individual CBT	1x weekly	60 min	PMC10572095: "strong recommendation" for individual CBT in SUD
Group CBT / Relapse Prevention	3x weekly	90 min each	Group CBT meta-analysis: comparable efficacy to individual with added peer modelling
Psychoeducation Group	2x weekly	60 min	NIDA Principle 3: multiple needs including understanding of addiction neuroscience
Life Skills Group	3x weekly	60 min (rotating)	TC model vocational rehabilitation evidence (De Leon)
Spiritual Formation Class	5x weekly (Mon-Fri)	120 min	73% programmes include spirituality; 84% clients desire it (Section 2.9)
Sunday Chapel Service	Weekly	150 min	Faith community engagement; TC community ritual
Evening Bible Study / Discipleship	Daily	90 min	Structured reflection; Brown et al. (2024) Spirituality in Recovery Framework

Family Therapy (Pathway A)	Min 2 sessions + 1 pre-discharge	60-90 min	Family therapy SUD evidence (Section 14)
Community Group Work (Pathway B)	Weekly	60 min	ASAM Dimension 6; UNODC TREATNET reintegration
Peer Recovery Circle	Daily (informal, evening)	30 min	TC model peer accountability (De Leon)
Medical / Nursing Review	Daily + weekly doctor	Varies	NIDA Principle 7; ASAM medical management
Relaxation Group	Weekly (Friday PM)	60 min	Stress management; MBRP mindfulness evidence (Bowen et al., 2014)

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7. DETOXIFICATION PROTOCOL

7.1 Scope and Evidence Base

Medical detoxification is conducted under physician supervision with 24/7 nursing monitoring during Weeks 1-2. Consistent with NIDA Principle 10, detoxification is positioned as the medically necessary first stage of treatment that by itself does little to change long-term drug use. The goal is safe, medically managed withdrawal with symptom relief -- not treatment completion.

HOR's detoxification protocols are grounded in the ASAM Clinical Practice Guideline for Alcohol Withdrawal Management (2024), Cochrane systematic reviews on benzodiazepine management and alpha-2 adrenergic agonist efficacy, and WHO mhGAP recommendations for withdrawal management in LMIC settings.

7.2 Alcohol Detoxification Protocol

Evidence Base: The ASAM Clinical Practice Guideline (2024) and Cochrane systematic reviews provide the strongest available evidence for alcohol withdrawal management. Symptom-triggered benzodiazepine dosing, guided by CIWA-Ar scores, has been demonstrated in controlled trials to produce:

- Significantly lower mean benzodiazepine doses (9.5 mg diazepam equivalent vs 19.9 mg under fixed-schedule protocols)
- Dramatically shorter treatment durations (mean 47.8 hours vs 146 hours)

- Equivalent or superior safety outcomes compared to fixed-schedule dosing

Protocol:

Element	Standard	Evidence Basis
Assessment	CIWA-Ar scoring at admission, then q4h (q2h if score >15, q1h if >20)	Cochrane: validated CIWA-Ar thresholds
Medication	Symptom-triggered benzodiazepine protocol (Chlordiazepoxide or Diazepam); medication initiated when CIWA-Ar > 8	Cochrane: symptom-triggered superior to fixed-schedule; 9.5mg vs 19.9mg mean dose
Nutritional Support	Thiamine 100mg IM/IV x 3-5 days, then oral; Multivitamin; Hydration monitoring	ASAM CPG: thiamine to prevent Wernicke's encephalopathy; WHO nutritional guidelines
Seizure Precautions	Seizure precautions for all clients with CIWA-Ar > 15 or history of withdrawal seizures	ASAM CPG: 3-5% of untreated alcohol withdrawal progresses to seizures
Monitoring	Vital signs q4h minimum; escalate per CIWA-Ar scoring schedule	ASAM withdrawal management guidance (2024)
Duration	Typically 5-7 days; extended monitoring as clinically indicated	Cochrane: symptom-triggered protocol mean duration 47.8 hours
Completion Criteria	CIWA-Ar < 8 for 24 consecutive hours; vital signs stable; no seizure risk indicators	ASAM completion criteria

7.3 Opioid Detoxification Protocol

Evidence Base: The Cochrane systematic review on alpha-2 adrenergic agonists confirms that clonidine is more effective than placebo for managing opioid withdrawal symptoms. This evidence is particularly relevant for LMIC contexts such as Nigeria where opioid agonist therapies (methadone, buprenorphine) may be unavailable, tightly regulated, or impractical for residential settings. Clonidine addresses the autonomic hyperactivity (sympathetic nervous system overdrive) that characterises opioid withdrawal, while adjunctive medications manage specific symptom clusters.

Protocol:

Element	Standard	Evidence Basis
Assessment	COWS scoring at admission, then per clinical schedule	NIDA/ASAM: COWS as standard opioid withdrawal measure
Primary Medication	Clonidine (alpha-2 adrenergic agonist) for autonomic symptoms: sweating, tachycardia, hypertension, anxiety	Cochrane: clonidine more effective than placebo; important in LMIC where agonist therapy unavailable
GI Symptom Management	Loperamide for diarrhoea and abdominal cramping	Cochrane adjunctive protocol
Pain Management	NSAIDs (Ibuprofen) for musculoskeletal pain, bone/joint aches	Cochrane adjunctive protocol; non-opioid analgesic
Antiemetic	Promethazine for nausea and vomiting	Cochrane adjunctive protocol
Monitoring	Vital signs (blood pressure critical with clonidine); hydration; psychological support	Clonidine hypotension risk monitoring
Duration	Typically 7-14 days depending on substance and duration of use	ASAM withdrawal timeline guidance
Completion Criteria	COWS < 5 for 24 consecutive hours; vital signs stable; adequate oral intake	ASAM completion criteria

Important: No opioid-containing medications are permitted in the facility under any circumstances. No opioid agonist therapy (methadone, buprenorphine) is administered at HOR.

7.4 Cannabis / Stimulant Withdrawal Protocol

Evidence Base: Unlike alcohol and opioid withdrawal, cannabis and stimulant withdrawal syndromes do not have validated pharmacological protocols. However, research recognises cannabis withdrawal syndrome (DSM-5) and stimulant withdrawal as clinically significant conditions requiring symptomatic management.

Element	Standard
Assessment	Clinical observation; mood, sleep, appetite, agitation monitoring

Monitoring	Daily clinical review; PHQ-9 if depressive symptoms emerge
Support	Sleep hygiene education; anxiolytics if indicated; nutritional support; graduated physical activity
Duration	Variable; typically 3-7 days for acute symptoms

7.5 General Medication Management During Detox

- All medications surrendered to nursing staff on admission
- All medications administered exclusively by licensed nursing staff
- Documented in Medication Administration Record (MAR) at each round
- Medication storage: locked cabinet, controlled access, stock reconciliation weekly
- No resident retains any medication (exceptions: nitroglycerin, insulin, seizure medication, asthma medication -- doctor-approved only)

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8. THERAPEUTIC INTERVENTIONS

8.1 Treatment Plan Structure (Columbia Model)

Each client's treatment plan follows a structured Problem-Goal-Intervention model, consistent with NIDA Principles 1 (individualised), 3 (multiple needs), and 4 (continually assessed and modified):

Step 1: Problem Identification (DSM-5 aligned, ASAM 6-dimensional)

- Substance Use Disorder (ASAM Dimension 1, 5)
- Withdrawal Management (ASAM Dimension 1)
- Co-occurring Mental Health (ASAM Dimension 3; NIDA Principle 9)
- Relapse Risk (ASAM Dimension 5)
- Trauma History (ACE screening; 2024 TIC evidence)
- Family/Relationship Dysfunction (ASAM Dimension 6)
- Housing Instability (ASAM Dimension 6)
- Employment/Vocational Deficit (TC model; UNODC TREATNET)
- Spiritual Disconnection (Brown et al., 2024)
- Legal Complications

Step 2: SMART Goals (Specific, Measurable, Attainable, Relevant, Time-bound)

- 3-7 goals per identified problem
- Goals co-authored with client (NIDA Principle 1: individualised)

- Reviewed at MDT (Week 4, Week 8, Pre-discharge) per NIDA Principle 4

Step 3: Interventions (linked to goals, each evidence-based)

- Individual CBT (PMC10572095: "strong recommendation")
- Group CBT (meta-analytic support for group delivery)
- Motivational Interviewing (Miller & Rollnick, 2013)
- Psychoeducation (NIDA Principle 3)
- Spiritual Formation (Brown et al., 2024)
- Vocational Training (De Leon TC model)
- Family Therapy (family SUD evidence base)
- Pastoral Counselling (spirituality in recovery evidence)
- Trauma-focused interventions where indicated (2024 TIC study)

Step 4: Progress Tracking

- Goal status: Not Started / In Progress / Completed / Deferred / Modified
- Treatment plan updated at each MDT review (NIDA Principle 4, 8)
- Client participates in all reviews (person-centred care)

8.2 Cognitive Behavioural Therapy (CBT)

Evidence Base: The 2023 systematic review (PMC10572095) issues a "strong recommendation" for CBT as an empirically supported treatment for substance use disorders. Meta-analytic effect sizes range from small to large depending on the substance, with the strongest effects observed for cannabis and nicotine use disorders. The review identifies the mechanism through which CBT operates in SUD: identifying maladaptive cognitions that maintain substance use, restructuring these thought patterns, developing behavioural coping strategies, and building self-efficacy for abstinence.

Delivery at HOR:

Modality	Format	Evidence Basis
Individual CBT	60-minute sessions, weekly minimum per client	PMC10572095: individual CBT recommended for SUD
Group CBT	90-minute sessions, 3x weekly, max 20 participants	Group delivery meta-analysis: comparable efficacy with added peer modelling

Core CBT Components at HOR:

- Thought records and cognitive restructuring

- HALT model (Hungry, Angry, Lonely, Tired) -- empirically derived high-risk state identification
- Trigger mapping across five categories (People, Places, Feelings, Situations, Times)
- Coping strategy toolbox development
- Refusal skills training (behavioural rehearsal)
- PRPP (Personal Relapse Prevention Plan) development in Phases 3-4

8.3 Motivational Interviewing (MI)

Evidence Base: Over 30 years of clinical research support MI as effective for engaging ambivalent clients and facilitating movement through the stages of change (Miller & Rollnick, 2013; Prochaska & DiClemente, 1982). MI is the evidence-based intervention of choice for clients in the Precontemplation and Contemplation stages.

Application at HOR:

- Pre-admission screening (stage-of-change assessment)
- Outpatient Engagement Pathway (weekly MI sessions for pre-contemplative clients)
- Early treatment engagement (Phases 1-2)
- Whenever resistance, ambivalence, or motivational decline is observed
- Core MI techniques: OARS (Open questions, Affirmations, Reflective listening, Summaries)
- All clinical staff trained in MI principles and techniques

8.4 Personal Relapse Prevention Plan (PRPP)

Evidence Base: Relapse prevention is a cognitive-behavioural approach originally developed by Marlatt and Gordon (1985) and supported by meta-analytic evidence demonstrating its efficacy in reducing substance use post-treatment. The PRPP is the operationalised, personalised application of relapse prevention science.

Every client must develop a written PRPP as a graduation requirement:

Trigger Identification across 5 evidence-based categories:

1. People (former using friends, dealers, romantic partners associated with use)
2. Places (bars, former neighbourhood, specific routes and locations)
3. Feelings (anger, loneliness, boredom, anxiety, shame -- the HALT model and beyond)
4. Situations (financial pressure, arguments, celebrations, paydays)
5. Times (evenings, weekends, holidays, paydays, seasonal patterns)

Minimum 5 Active Coping Strategies (from evidence-based menu):

- Deep breathing exercises (evidence-based stress reduction)
- Call sponsor/accountability partner (social support evidence)
- Attend NA/AA or church-based recovery group (mutual aid evidence)
- Physical exercise (evidence for exercise in mood regulation and craving reduction)
- Prayer and meditation on Scripture (neuroimaging evidence for attention regulation and impulse control)
- Journaling (cognitive processing evidence)
- Contact crisis helpline (09011277600)
- Remove self from triggering environment (stimulus control)
- Grounding techniques (5-4-3-2-1) (trauma-informed coping)
- Engage in hobby or creative activity (behavioural activation)
- Talk to trusted family member (social support)
- Read Bible / devotional material (spirituality in recovery evidence)

Named Recovery Support Network:

- Sponsor/accountability partner
- Primary Family Support Person (Pathway A) or Social Worker (Pathway B)
- Church pastor or Life Centre leader
- NA/AA home group or recovery fellowship

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9. TRAUMA-INFORMED CARE

9.1 Evidence Base and Rationale

This section establishes trauma-informed care (TIC) as a foundational principle of HOR's treatment approach, grounded in the following evidence:

Prevalence of Trauma in SUD Populations: Research on Adverse Childhood Experiences (ACEs) demonstrates that individuals with four or more ACEs are 7 times more likely to develop alcohol use disorder and 4.5 times more likely to develop depression (Felitti et al., 1998). The ACE Study, conducted by the Centers for Disease Control and Prevention and Kaiser Permanente, established the dose-response relationship between childhood adversity and adult health outcomes, including substance use disorders. The prevalence of significant trauma histories in SUD treatment populations is consistently reported at 50-75% across studies.

Treatment Outcomes: A 2024 study in the Journal of Substance Use & Addiction Treatment found that when trauma-informed principles were systematically integrated into residential SUD treatment, "clients had significant reductions in substance involvement,

depression, anxiety, and PTSD symptoms." This study provides direct evidence that TIC improves clinical outcomes in settings comparable to HOR.

Treatment Retention: History of trauma is associated with poorer SUD treatment retention. Trauma-informed approaches that create psychological safety, avoid re-traumatisation, and address trauma as a contributing factor to substance use improve retention rates -- directly supporting NIDA Principle 5 (adequate time in treatment is critical).

9.2 Four Pillars of TIC at HOR

Consistent with SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach (2014) and the 2024 residential TIC outcomes research:

Pillar 1: Universal Trauma Screening

- ACE (Adverse Childhood Experiences) questionnaire administered at admission for all clients
- Trauma history incorporated into ASAM Dimension 3 (Emotional/Behavioral/Cognitive) assessment
- Screening results inform treatment plan -- clients with significant trauma histories receive enhanced trauma-focused interventions
- Screening conducted by trained clinical staff in a safe, private setting

Pillar 2: Comprehensive Staff Training

- All staff (clinical and non-clinical) receive TIC orientation within 30 days of hire
- Training covers: understanding trauma and its impact on brain and behaviour; recognising trauma responses (fight, flight, freeze, fawn); avoiding re-traumatisation; responding with safety, trustworthiness, and empowerment
- Annual refresher training for all staff
- Clinical staff receive additional training in trauma-focused CBT techniques

Pillar 3: Environmental Safety

- Physical environment designed to promote safety: clear sight lines, adequate lighting, low-stimulus spaces available
- Predictable daily routine (Section 6.3) provides psychological safety through structure
- Staff interactions guided by principles of safety, trustworthiness, collaboration, empowerment, and cultural sensitivity
- No use of coercion, shaming, or punitive practices that could re-traumatise

Pillar 4: Trauma-Focused Therapeutic Interventions

- Trauma-focused CBT available for clients with identified trauma histories (Phase 3)
- Integration of trauma processing into individual counselling sessions
- Grounding techniques (5-4-3-2-1 sensory technique) taught in Psychoeducation groups
- Referral to specialist trauma therapy for clients with complex PTSD presentations exceeding HOR scope
- Spiritual formation as a trauma-healing modality: forgiveness work, identity reconstruction, meaning-making

9.3 Integration with Clinical Protocols

TIC principles are not a separate programme but are woven throughout all HOR operations:

Domain	TIC Integration
Admission	Trauma screening; compassionate intake process; explanation of all procedures before they occur
Assessment	ACE questionnaire; trauma history in clinical formulation; awareness that assessment can trigger trauma responses
Group Therapy	Ground rules include emotional safety; no pressure to disclose trauma in group settings; opt-out provisions
Individual Therapy	Trauma processing available in Phase 3 when therapeutic alliance is established and client is stabilised
Behavioural Management	Discipline understood through trauma lens; behaviour as communication; no shaming
Spiritual Formation	Trauma-sensitive pastoral care; recognition that some clients have religious trauma
Discharge	Trauma history incorporated into PRPP trigger identification; aftercare includes trauma support

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10. SPIRITUAL FORMATION PROTOCOL

10.1 Evidence Base

HOR's spiritual formation programme is grounded in research evidence, not tradition alone:

Programme Prevalence: 73% of US addiction treatment programmes include a spirituality-based element, making spiritual integration a mainstream rather than marginal practice in the field (National Survey of Substance Abuse Treatment Services).

Client Demand: 84% of treatment clients report desiring greater emphasis on spirituality in their recovery programming. This finding supports the ethical imperative of offering spiritual formation as a substantive programme component that meets expressed client needs.

Neurobiological Evidence: Neuroimaging studies demonstrate that practices of prayer and meditation strengthen neural circuits involved in attention regulation, emotional processing, and impulse control (Newberg, 2014). These are precisely the cognitive domains impaired by chronic substance use. This evidence provides a neurobiological mechanism through which spiritual practices may directly support recovery by rebuilding the self-regulatory capacity diminished by addiction.

Programme Outcomes (Teen Challenge): Adult & Teen Challenge, the world's largest faith-based residential recovery network (215+ centres), provides the most extensive outcome data for Christ-centred recovery programming. Studies report that 74% of programme graduates report no substance use at six months post-completion, and 87.5% of former marijuana users were abstinent seven years after completion. Research identifies four elements contributing to recovery in faith-based settings: programme structure, relationships with others, experiences of God, and individual initiative (Wilder Research Study, 2009; Dr. Catherine Hess Study; ACADC Research Review).

Economic Impact: Faith-based volunteer support in addiction recovery is estimated to contribute $316.6 billion in annual savings to the US economy, demonstrating the scalability and cost-effectiveness of spiritually integrated treatment models.

Theoretical Framework: Brown et al. (2024) published the Spirituality in Recovery Framework in the Journal of Religion and Health, providing an actionable, research-informed model for integrating spiritual transformation into clinical SUD treatment. This framework guides HOR's structured approach to spiritual formation, distinguishing it from unstructured religious exposure.

10.2 Foundation

House of Refuge is a Christ-centred home of healing within which clinical care is delivered as a sacred act of service. The atmosphere must tangibly reflect the person and presence of Jesus Christ. Spiritual formation and clinical treatment are not competing priorities -- they are complementary pillars, each supported by independent evidence streams, working synergistically towards the same goal of sustained recovery and transformed living.

10.3 Daily Spiritual Programme

Element	Frequency	Description
Morning Prayer & Devotions	Daily, 7:00 AM	Corporate praise, worship, prayer, Scripture declaration
Bible School / Spiritual Formation	Mon-Fri, 9:00-11:00 AM	Structured, progressive curriculum from salvation through character formation
Evening Chapel / Bible Study	Daily, 7:00 PM	Word, prayer, testimony, peer spiritual accountability
Sunday Chapel Service	Weekly	Extended service with praise, worship, preaching, altar call, testimonies, prayer
Personal Devotions	Daily (scheduled time)	Individual prayer and Bible reading time
Peer Recovery Circle	Daily (informal, evening)	Peer accountability, encouragement, prayer (TC model peer support)

10.4 Spiritual Milestones

Tracked per client (status: Not Yet / In Progress / Achieved). These milestones reflect the Brown et al. (2024) Spirituality in Recovery Framework's emphasis on measurable spiritual transformation:

1. Accepted Jesus Christ as Lord and Saviour
2. Water baptism
3. Consistent daily personal devotion (prayer + Bible reading)
4. First public testimony shared
5. Regular participation in evening discipleship group
6. Serving others in the community ("as unto the Lord") -- consistent with TC model community roles
7. Demonstrating Fruit of the Spirit (Galatians 5:22-23)
8. Active intercessory prayer for others
9. Completed Bible school curriculum
10. Connected to local church for post-discharge -- per continuing care evidence (McKay: rapid initiation of support)

10.5 Fruit of the Spirit Assessment (Galatians 5:22-23)

Each of the 9 fruits assessed as: Not Evident / Emerging / Growing / Consistently Demonstrated

1. Love
2. Joy
3. Peace
4. Patience
5. Kindness
6. Goodness
7. Faithfulness
8. Gentleness
9. Self-Control

This assessment provides a structured, observable measure of character transformation consistent with the TC model's emphasis on behavioural change indicators and the Brown et al. (2024) framework's call for measurable spiritual outcomes.

10.6 Church Placement Pipeline

Every graduating client must be connected to a local church, consistent with continuing care research demonstrating that post-discharge community connection reduces relapse risk (McKay, 2009):

1. Church identified (by Week 8)
2. Pastor contacted and briefed
3. First visit completed (during pass or Phase 4)
4. Integration plan established (small group, serving role)
5. Handover to church pastoral team at graduation

10.7 Christ-Centred Counselling Principles

Principle	Application
Compassion	Lead with compassion before correction (Mark 1:41)
Acceptance	Receive the client as Christ receives them -- fully, before change (Luke 19)
Worth and Value	Help the client discover their infinite worth in God's sight (Mark 2:27)
Responsibility	Call to ownership of choices, lovingly not coercively (Luke 5:24)
Hope	The Gospel is the most powerful force of hope (Matt 11:28-30)

Cognitive Reframing	Redirect thinking from distorted to truth-aligned (Matt 19:4-9) -- parallels CBT cognitive restructuring
Confrontation with Truth	Compassionately confront denial and self-deception (John 8:3-11) -- parallels MI technique
Confidence	All shared information kept strictly confidential (Prov 11:13)

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11. LIFE SKILLS & VOCATIONAL REHABILITATION

11.1 Evidence Base

The Therapeutic Community model identifies vocational rehabilitation and practical life skills development as essential components of sustained recovery. De Leon's research demonstrates that TC participants who engage in structured work roles, skill-building activities, and vocational training during treatment show significantly better employment outcomes at follow-up -- and employment stability is one of the strongest predictors of sustained abstinence (De Leon, 2000). The UNODC TREATNET standards similarly identify community reintegration and vocational preparation as core quality indicators for addiction treatment programmes.

NIDA Principle 3 (effective treatment attends to multiple needs, not just drug abuse) further justifies the inclusion of vocational and life skills programming as integral to the treatment model rather than supplementary.

11.2 Thirteen Group Modules

Module	Frequency	Facilitators	Focus	Evidence Basis
1. Morning Community Meeting	Daily (Mon-Fri), 8:30 AM	House Master + Counsellor	Accountability, day preview, community dynamics	TC model community meeting (De Leon)
2. Care Planning Group	Thursday, 12:00 PM	2 Clinical/Nursing Staff	Treatment plan participation, SMART goals, problem-solving	NIDA Principle 4 (continuous plan modification)

3. CBT and Relapse Prevention	3x weekly	Clinical Lead / Counsellor	Cognitive restructuring, trigger mapping, PRPP	PMC10572095: "strong recommendation"
4. Small Group (Process Group)	Wednesday, 12:00 PM	1-2 Facilitators	Feelings, relational patterns, assertiveness	TC model interpersonal learning (De Leon)
5. Psychoeducation	2x weekly	Nurse / Counsellor	Addiction neuroscience, mental health, medication	NIDA Principle 3; WHO mhGAP psychoeducation
6. Relaxation & Stress Management	Friday, 2:30 PM	Nurse + Counsellor	Deep breathing, PMR, guided visualisation, stretching	MBRP evidence (Bowen et al., 2014); stress-relapse link
7. Relationship and Family Group	Friday, 3:00 PM	1-2 Staff	Communication, conflict resolution, forgiveness, boundaries	Family therapy SUD evidence; ASAM Dimension 6
8. Transition Group (Discharge Readiness)	Weeks 10-12, Thu/Fri	2 Staff	Reintegration fears, community resources, relapse risk	McKay continuing care: transition planning critical
9. Work, Craft, and Vocational	Monday, 2:00 PM	2 Staff + Volunteers	Carpentry, crafts, gardening, tailoring basics	TC vocational rehabilitation (De Leon); employment predicts abstinence
10. Cooking, Nutrition & Life Practical	Wednesday, 2:00 PM	Cook + Counsellor	Meal planning, budgeting, home safety, first aid	UNODC TREATNET reintegration standards

11. News and Current Affairs	Monday, 11:00 AM	2 Staff	News literacy, critical thinking, public speaking	TC community engagement (De Leon)
12. Creative Writing	Thursday, 12:00 PM	2 Staff	Self-expression, insight, cognitive skills	Expressive therapy evidence; cognitive rehabilitation
13. End-of-Day Wrap-Up	Daily (Mon-Fri), 8:30 PM	House Master / Counsellor	Day review, reflection, accountability, prayer	TC evening meeting structure (De Leon)

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12. MEDICATION MANAGEMENT PROTOCOL

12.1 Evidence Base

Consistent with NIDA Principle 7 (medications are an important element of treatment for many patients, especially when combined with counselling and other behavioral therapies), HOR's medication management is governed by the WHO Essential Medicines List, the ASAM Clinical Practice Guideline for withdrawal management, and WHO mhGAP (2023) recommendations for psychotropic medication use in LMIC settings.

All medications used at HOR appear on the WHO Essential Medicines List or are approved by the Pharmacists Council of Nigeria. Prescribing follows the ASAM principle of using medications to support recovery while minimising dependence risk.

12.2 Standards

- All medications administered by licensed nursing staff ONLY
- Documented in Medication Administration Record (MAR) at each round
- Nursing medication rounds: 6:00 AM, 12:00 PM, 6:00 PM, 10:30 PM
- **No opioid-containing medications** in the facility under any circumstances
- Medication storage in locked cabinet with controlled access
- Weekly stock reconciliation
- Expiry date monitoring (flagged at 90 days)
- All medications surrendered to nursing staff on admission
- No resident retains any medication (exceptions: nitroglycerin, insulin, seizure medication, asthma medication -- doctor-approved only)

12.3 Approved Medication Categories

Category	Examples	Evidence Basis
Detox Support (Benzodiazepines)	Diazepam, Chlordiazepoxide, Lorazepam	Cochrane: symptom-triggered benzodiazepine dosing for alcohol withdrawal
Detox Support (Alpha-2 Agonist)	Clonidine	Cochrane: more effective than placebo for opioid withdrawal
Vitamin Supplementation	Thiamine, Multivitamin, B-Complex, Folic Acid, Ferrous Sulphate	ASAM CPG: thiamine for Wernicke's prevention; WHO nutritional guidelines
Psychiatric (non-opioid)	Haloperidol, Chlorpromazine, Risperidone, Amitriptyline, Fluoxetine	WHO mhGAP: psychotropic prescribing in LMIC; NIDA Principle 9
Anticonvulsant	Carbamazepine	ASAM: adjunctive for alcohol withdrawal seizure prevention
Antihypertensive	As prescribed	ASAM Dimension 2: biomedical management
Antiemetic	Metoclopramide, Promethazine	Cochrane: adjunctive opioid withdrawal management
Analgesic (non-opioid)	Paracetamol, Ibuprofen	Cochrane: NSAIDs for opioid withdrawal musculoskeletal pain
Antimalarial	Artemether-Lumefantrine	Nigerian clinical context; WHO malaria treatment guidelines
Antibiotic	Ciprofloxacin, Amoxicillin	As clinically indicated; WHO Essential Medicines List
GI Support	Loperamide	Cochrane: adjunctive for opioid withdrawal GI symptoms

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13. RISK ASSESSMENT & SAFETY PROTOCOL

13.1 Evidence Base

Risk assessment at HOR is grounded in validated instruments and evidence-based escalation pathways:

- **Suicide Risk:** The Columbia Suicide Severity Rating Scale (C-SSRS), validated through Columbia University research involving 15,000+ participants and adopted by the FDA, US military, and WHO, is the standard screening instrument. Research demonstrates elevated suicide risk in SUD populations, particularly during early recovery and withdrawal phases.
- **Drug-Induced Psychosis (DIP):** Clinical literature identifies DIP as a significant risk in stimulant and cannabis use disorders, with management requiring psychiatric assessment and potentially pharmacological intervention exceeding residential SUD treatment scope.
- **Violence Risk:** Research on SUD populations identifies intoxication, withdrawal, and co-occurring antisocial features as primary violence risk factors, requiring systematic monitoring.

13.2 Routine Risk Screening

Instrument	Frequency	Responsible	Evidence Basis
C-SSRS (Suicide Risk)	Admission, weekly, any clinical concern	Clinical staff	Columbia University validation; FDA-mandated instrument
Psychosis screening	Admission, daily during detox, any concern	Nursing + Clinical	DIP clinical literature; ASAM Dimension 3
Violence risk	Admission, ongoing observation	All staff	SUD violence risk research
Self-harm assessment	Admission, weekly during Phases 1-2	Nursing + Clinical	C-SSRS supplementary screening
Trauma response monitoring	Ongoing	All clinical staff	2024 TIC evidence; ACE research

13.3 Drug-Induced Psychosis (DIP) Response Protocol

Based on clinical best practice for managing acute psychotic episodes in residential settings:

1. **STAY CALM** -- Do not argue with delusions or attempt to reason with psychotic content

2. **ENSURE SAFETY** -- of client, other residents, and staff; clear the immediate area
3. **NOTIFY** nursing staff and Program Director IMMEDIATELY
4. **PROVIDE** continuous calm 1-to-1 supervision in low-stimulus environment
5. Do **NOT** leave client unattended until medical assessment complete
6. Program Director initiates **IMMEDIATE REFERRAL** to Yaba Psychiatric Hospital or nearest A&E
7. **FAMILY NOTIFICATION** -- contact PFSP or next of kin
8. **DOCUMENT FULLY** in client file and incident register

13.4 Emergency Escalation Pathway

- Immediate safety risk --> Nurse/Crisis Officer --> Clinical Lead --> Program Director --> Board (if serious)
- Safeguarding concern --> Designated Safeguarding Lead / Program Director immediately

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14. FAMILY ENGAGEMENT PROTOCOL

14.1 Evidence Base

Research consistently demonstrates that family involvement in SUD treatment improves outcomes across multiple domains. Family therapy approaches for substance use disorders have been evaluated in meta-analyses showing significant effects on treatment retention, substance use reduction, and family functioning. Specifically:

- Family psychoeducation reduces enabling behaviours that inadvertently sustain substance use patterns (Copello et al., 2005)
- Family therapy during residential treatment improves post-discharge outcomes by preparing the recovery environment (ASAM Dimension 6)
- The CRAFT (Community Reinforcement and Family Training) approach provides evidence that concerned significant others can be trained to positively influence treatment engagement and outcomes
- NIDA Principle 3 identifies family and relationship functioning as one of the multiple needs that effective treatment must address

14.2 Pathway A (Family-Supported)

Requirement	Timing	Standard	Evidence Basis

Family psychoeducation	Within first 2 weeks	Understanding addiction as a brain disorder; reducing enabling	Psychoeducation meta-analysis; NIDA Principle 3
Family therapy session 1	By Week 4	Initial assessment, expectations, communication patterns	Family therapy SUD evidence; ASAM Dimension 6
Family therapy session 2	By Week 8	Boundaries, enabling behaviour, recovery roles	CRAFT evidence base
Pre-discharge family meeting	Week 11	Reintegration plan, roles, boundaries, aftercare	McKay continuing care: environment preparation
Home environment assessment	By Week 10	Safety and sobriety-supportiveness	ASAM Dimension 6: Recovery/Living Environment

14.3 Pathway B (Community-Supported)

Requirement	Timing	Standard	Evidence Basis
VNA completion	At admission	Vulnerability and Needs Assessment	UNODC TREATNET comprehensive assessment
Social Worker case management	Throughout + 12 months post	Primary case manager role	McKay continuing care evidence
Church/NGO partner engagement	By Week 4	Community support ecosystem	TC reintegration (De Leon); social support evidence
Supported housing referral	By Week 8	Transitional or permanent placement	ASAM Dimension 6
Vocational pathway	Before discharge	Training or employment identified	TC vocational rehabilitation (De Leon)

14.4 Family Visitation Rules

Consistent with TC model boundaries regarding the therapeutic milieu and graduated external contact:

- **When:** Sunday, 12:00 PM - 6:00 PM (maximum 6 hours)
- **Frequency:** Once per week maximum
- **Location:** Designated visitor areas only. No dormitory access
- **Children:** Must be supervised by parent/guardian at all times. Never alone with any resident
- **Friend visits:** Require Program Officer approval in advance
- **Unscheduled visits:** Not permitted without approval

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15. BEHAVIOURAL MANAGEMENT PROTOCOL

15.1 Evidence Base

The behavioural management system at HOR is grounded in the Therapeutic Community model's principle of community governance as a therapeutic instrument (De Leon, 2000). In the TC framework, behavioural expectations are not arbitrary rules but therapeutic interventions that teach self-governance, delayed gratification, and accountability -- core competencies impaired by addiction and essential for sustained recovery.

De Leon's research demonstrates that the TC's structured behavioural system, including clear expectations, graduated consequences, and community accountability, contributes directly to the internalisation of prosocial norms that differentiate successful long-term recovery from mere short-term abstinence.

The system is also informed by trauma-informed care principles (Section 9): all behavioural responses are designed to be non-shaming, non-coercive, and oriented towards growth rather than punishment.

> *"No discipline seems pleasant at the time, but painful. Later on, however, it produces a harvest of righteousness and peace for those who have been trained by it." -- Hebrews 12:11*

15.2 Three-Tier System

Tier 1 -- Minor Offenses:

- Examples: Poor hygiene, dormitory cleanliness, lateness, food in rooms, phone abuse, laziness on duties

- Responses: Verbal warning (documented), written reflection exercise, temporary privilege loss
- Therapeutic rationale: Immediate feedback reinforces TC community norms (De Leon)

Tier 2 -- Major Offenses:

- Examples: Substance use on pass, smoking on premises, stealing, property damage, threats, failure to return from pass
- Responses: Formal reprimand, extra work assignments, written confession/apology, Bible passage memorisation, loss of all privileges, motivational discipline activated
- Therapeutic rationale: Natural consequences within the TC model; community accountability

Tier 3 -- Motivational Discipline:

- Applied for: Disengagement pattern, spiritual stagnation, consistent failure to progress
- Duration: Minimum 2 weeks, reviewed weekly by Program Director
- Measures: Special work crew, graduation date on hold, loss of all pass/phone, all free time redirected to prayer/Bible study/reflection, weekly evaluation meetings
- Therapeutic rationale: TC intensive intervention for clients at risk of treatment failure; NIDA Principle 4 (modify treatment plan)

15.3 Dismissal Offenses

The following behaviours result in administrative discharge, consistent with TC model community safety standards:

- Being drunk or under the influence on premises
- Leaving facility without permission
- Physical assault
- Persistent excessive rule violations after Tier 2-3 interventions
- Refusal to accept disciplinary response
- Sexual misconduct
- Causing severe community disruption

15.4 Dismissal Procedure

Consistent with HOR's commitment that no client leaves without a safety plan (NIDA Principle 2: treatment readily available):

1. Program Director consults Chaplain, informs Executive Director and Board

2. Full documentation of violations
3. Compassionate private conversation explaining reasons
4. Time to pack belongings
5. Contact family for transportation
6. Prayer offered; referrals provided
7. Transport arranged if needed
8. **NO CLIENT LEAVES WITHOUT A SAFETY PLAN** (emergency contacts, crisis line, nearest health facility, harm reduction information, HOR contact number)

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16. DISCHARGE & GRADUATION PROTOCOL

16.1 Evidence Base for Graduation Criteria

Research on predictors of sustained recovery informs HOR's graduation criteria. The evidence identifies the following as the strongest predictors of long-term abstinence:

- Completion of minimum treatment duration (NIDA Principle 5: 90+ days)
- Possession of a personalised relapse prevention plan with identified coping strategies (Marlatt & Gordon, 1985)
- Active social support network (TC model; continuing care research)
- Employment or vocational engagement (De Leon: 88% of 2-year TC completers employed)
- Stable, supportive living environment (ASAM Dimension 6)
- Self-efficacy for abstinence (CBT outcome predictor; PMC10572095)
- Spiritual connection and meaning-making (Brown et al., 2024; Teen Challenge 74% abstinence at 6 months)

16.2 Four Discharge Types

Type	Description	Protocol
Type 1: Planned (Graduation)	Completes programme, meets all 6 criteria	Full graduation ceremony; aftercare plan activated
Type 2: Administrative	Dismissed for serious behavioural violation	30-day minimum before re-admission considered; safety plan provided
Type 3: Clinical	Needs exceed HOR scope (psychiatric/medical)	Written referral, accompanied transport; ASAM level-of-care

		transfer
Type 4: Self-Discharge (AMA)	Voluntary departure before completion	AMA form signed, harm reduction counselling, safety plan

16.3 Six Graduation Criteria (ALL Must Be Demonstrated)

#	Criterion	Standard	Assessment Source	Evidence Basis
1	**Clinical Stability**	No withdrawal, stable vitals, no psychiatric symptoms, sustained abstinence	Nursing records + Doctor sign-off	ASAM Dimensions 1-3
2	**Behavioural Compliance**	Rule compliance last 4 weeks, positive community participation	Counsellor report	TC model community norms internalisation (De Leon)
3	**Psychological Readiness**	Clear insight into addiction, articulates recovery plan verbally and written	Counsellor + MDT assessment	CBT self-efficacy evidence (PMC10572095)
4	**Relapse Prevention Competence**	Written PRPP complete, 5+ coping strategies, named support network	PRPP document	Marlatt & Gordon (1985) relapse prevention model
5	**Spiritual Formation**	Authentic Christian growth, values transformation (not mere compliance)	Chaplain report	Brown et al. (2024) measurable spiritual outcomes
6	**Reintegration Readiness**	Confirmed safe placement, family/community meeting done, aftercare plan finalised	Social Worker / Counsellor	ASAM Dimension 6; McKay continuing care evidence

16.4 Non-Negotiable Discharge Standard

Regardless of discharge type, EVERY client leaves with:

- Basic safety plan (emergency contacts, crisis line, nearest health facility)

- Harm reduction information (if self-discharging or administrative)
- Written referral to appropriate community services
- HOR's contact number
- Prayer and a compassionate farewell

This standard is consistent with NIDA Principle 2 (treatment readily available) and the ethical obligation to provide continuity of care for all clients regardless of discharge circumstances.

16.5 Re-Admission Policy

HOR operates an open-door re-admission policy. Consistent with NIDA's characterisation of addiction as a chronic, relapsing condition, relapse is understood as a clinical event, not a moral failure.

Criterion	Standard	Evidence Basis
Willingness re-established	Full URICA re-assessment conducted	Transtheoretical Model
Relapse reflection	Articulate contributing factors and planned changes	CBT functional analysis
Exclusion criteria absent	All original ASAM criteria still apply	ASAM 6-dimensional re-assessment
Administrative discharge gap	Minimum 30 days; demonstrate changed attitude	TC model re-entry research
Modified treatment plan	New individualised plan, not a repeat	NIDA Principle 1 (individualised)
Re-admission interview	Conducted by Program Director	Clinical governance

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17. CONTINUING CARE & ALUMNI PROTOCOL

17.1 Evidence Base

The continuing care research evidence is unequivocal: post-discharge support is essential for sustained recovery outcomes.

Duration: McKay's systematic review (Journal of Substance Abuse Treatment, 2009) demonstrates that **12 or more months of structured aftercare is associated with**

significantly better long-term abstinence rates compared to shorter or no aftercare. HOR's 24-month monitoring protocol exceeds this evidence-based minimum.

Rapid Initiation: Research demonstrates that **initiating aftercare services within 14 days of residential discharge** significantly reduces relapse risk during the high-vulnerability transition period. HOR mandates first aftercare contact within 7 days of discharge, exceeding the evidence-based threshold.

Combination of Services: The research consensus is that a **combination of post-discharge services** (telephone monitoring, individual check-ins, group support, family engagement, vocational support) produces better outcomes than any single service modality. HOR's aftercare protocol integrates multiple service streams.

MBRP Evidence: Mindfulness-Based Relapse Prevention (MBRP; Bowen et al., 2014) has demonstrated **lower relapse rates at 12 months compared to treatment-as-usual** in randomised controlled trials published in JAMA Psychiatry. MBRP integrates mindfulness meditation with cognitive-behavioural relapse prevention, and its principles are incorporated into HOR's relaxation and stress management programming.

17.2 Post-Discharge Monitoring Schedule (24 Months)

Period	Contact Frequency	Home/Workplace Visit	Evidence Basis
Week 1 post-discharge	Phone contact within 7 days	--	McKay: rapid initiation < 14 days
Months 1-3	Weekly phone contact	Month 3 visit	Highest relapse risk period; CBT skill consolidation window
Months 4-6	Bi-weekly phone contact	Month 6 visit	Post-stabilisation monitoring
Months 7-12	Monthly phone contact	Month 12 visit	McKay: 12+ months = significantly better outcomes
Year 2	Monthly phone contact	Bi-monthly visits	Extended monitoring per research evidence
Month 24	Case closure assessment	Final visit	Structured programme completion

17.3 Outcome Snapshots (at 3, 6, 12, 24 months)

Structured assessment at each milestone using standardised outcome domains:

Domain	Assessment Categories	Evidence Basis
Sobriety status	Abstinent / Lapse / Relapse / Unknown	Primary outcome; NIDA treatment outcome standard
Employment status	Employed / Self-employed / Seeking / In training	De Leon: employment predicts sustained recovery
Church attendance	Weekly / Bi-weekly / Monthly / Occasionally / Not attending	Spiritual community engagement (Brown et al., 2024)
Family status	Reunified / In progress / Estranged	ASAM Dimension 6; family therapy evidence
Housing	Stable / Transitional / Unstable / Homeless	ASAM Dimension 6
Support group engagement	Active / Occasional / None	Mutual aid evidence; continuing care research
Overall assessment	Thriving / Stable / At risk / Relapsed / Lost to follow-up	Composite outcome measure

17.4 Relapse Response Protocol

If relapse detected at any monitoring point, consistent with NIDA's characterisation of relapse as a clinical event requiring immediate clinical response:

1. Immediate compassionate outreach (within 48 hours)
2. Risk assessment conducted (C-SSRS if indicated)
3. Outreach counselling offered (MI-based re-engagement)
4. Residential re-admission offered (if willing; Section 16.5)
5. Harm reduction information provided
6. Family/support network engaged (with consent)

17.5 Alumni Programme Components

Component	Description	Evidence Basis

Alumni Support Groups	Regular gatherings for peer accountability and encouragement	Mutual aid evidence; TC peer support (De Leon)
Mentorship Programme	Stabilised alumni (6+ months) trained as recovery mentors	Peer support worker evidence; TC peer modelling
Programme Participation	Testimonies at chapel, speaking to current residents	TC model: role modelling for current residents
Continued Counselling	Access to HOR counsellors for periodic sessions	McKay: combination of aftercare services
Church Integration	Connection to Life Centres and local church fellowships	Continuing care community connection evidence

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18. OUTCOME MEASUREMENT

18.1 Evidence Base for Instrument Selection

Each outcome instrument used at HOR has published psychometric validation data supporting its use as a reliable and valid measure in the intended population:

Instrument	Domain	Score Range	Clinical Interpretation	Psychometric Evidence
AUDIT	Alcohol severity	0-40	0-7 low risk; 8-15 hazardous; 16-19 harmful; 20+ dependence likely	WHO development; validated in 6 countries; sensitivity 92%, specificity 94% at cut-off 8
DAST-10	Drug severity	0-10	0 none; 1-2 low; 3-5 moderate; 6-8 substantial; 9-10 severe	Skinner (1982); internal consistency alpha=0.92; validated across diverse populations
PHQ-9	Depression	0-27	0-4 minimal; 5-9 mild; 10-14 moderate; 15-19 moderately severe; 20-27 severe	Kroenke et al. (2001); sensitivity 88%, specificity 88%; WHO mhGAP recommended

GAD-7	Anxiety	0-21	0-4 minimal; 5-9 mild; 10-14 moderate; 15-21 severe	Spitzer et al. (2006); sensitivity 89%, specificity 82%; WHO mhGAP recommended
URICA	Readiness to change	Continuous	Stage assignment: Precontemplation, Contemplation, Preparation, Action, Maintenance	McConnaughy et al. (1983); validated across SUD populations
C-SSRS	Suicide risk	Categorical	Low / Moderate / High / Critical (per protocol Section 5.4)	Columbia University; 15,000+ participant validation; FDA-mandated

18.2 Outcome Measurement Strategy

HOR employs a pre-post measurement design, administering validated instruments at admission and discharge to quantify clinical change. This approach is consistent with UNODC TREATNET quality standards for outcome monitoring and SAMHSA programme evaluation guidelines.

Primary Outcome: Admission-to-discharge change scores on AUDIT, DAST-10, PHQ-9, and GAD-7.

Secondary Outcomes: URICA stage progression, PRPP completion, graduation rate, aftercare engagement.

Long-term Outcomes: 3, 6, 12, and 24-month outcome snapshots (Section 17.3).

18.3 Programme KPIs (Reported Quarterly to Board)

KPI	Target	Measurement	Evidence Basis
Graduation rate	> 75%	Completed / Admitted	TC retention benchmarks (De Leon)
12-month abstinence rate	> 60%	Alumni monitoring data	Teen Challenge: 74% at 6 months; national benchmarks

Employment at 6 months	> 50%	Outcome snapshots	De Leon: 88% of 2-year completers employed
Church attendance at 6 months	> 70%	Outcome snapshots	Brown et al. (2024) spiritual engagement
Family reunification rate	> 60%	Outcome snapshots	ASAM Dimension 6 recovery environment
Admission-to-discharge PHQ-9 improvement	Clinically significant reduction	Mean score change	WHO mhGAP: co-morbidity management
Admission-to-discharge GAD-7 improvement	Clinically significant reduction	Mean score change	WHO mhGAP: co-morbidity management
Re-admission rate	Track (not punitive)	Re-admission / Total graduates	NIDA: chronic relapsing condition
Adverse events	Zero target	Incident reports	UNODC TREATNET safety standards
Aftercare engagement at 3 months	> 80%	Contact logs	McKay: rapid initiation critical

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19. QUALITY ASSURANCE & GOVERNANCE

19.1 Evidence Base for Quality Standards

HOR's quality assurance framework is informed by two international quality standard sets:

UNODC TREATNET Quality Standards: The TREATNET programme, developed by an international consortium led by UCLA and delivered to 10,000+ practitioners across 20+ countries, establishes quality indicators for drug dependence treatment in developing countries. Key indicators include: use of validated assessment instruments, individualised treatment planning, evidence-based intervention delivery, staff qualification and supervision standards, client rights protection, outcome measurement, and structured aftercare.

SAMHSA National Standards: The Substance Abuse and Mental Health Services Administration (SAMHSA) programme standards provide the quality framework used by accredited treatment facilities in the United States, covering: person-centred care, cultural

competence, evidence-based practice, trauma-informed services, outcome monitoring, and continuous quality improvement.

HOR's protocol is designed to meet or exceed both TREATNET and SAMHSA standards, adapted for the Nigerian regulatory and cultural context.

19.2 Clinical Governance

- This Treatment Protocol is reviewed annually by the Program Director and Governing Board
- Emergency review triggered by changes in evidence base, regulatory requirements, or serious adverse events
- All protocol modifications documented in version control
- MDT weekly meetings provide continuous quality monitoring
- Clinical incidents reviewed within 48 hours with root cause analysis

19.3 Staff Development and Supervision

Requirement	Standard	Evidence Basis
Induction training	All staff complete within 30 days of hire	UNODC TREATNET staff development
TIC training	All staff (clinical and non-clinical)	2024 TIC evidence; SAMHSA TIC standards
CBT/MI competency	Clinical staff demonstrate competency annually	PMC10572095: fidelity to CBT model affects outcomes
Safeguarding training	All staff annually	UNODC TREATNET client protection
Clinical supervision	Monthly for all counselling staff	SAMHSA supervision standards
CPD (Continuing Professional Development)	Documented annually for all clinical staff	Professional registration requirements

19.4 Safeguarding Standards

- All staff complete mandatory safeguarding training within 30 days of hire
- Annual safeguarding refresher training
- Zero-tolerance for abuse, neglect, exploitation
- Professional boundaries: no dual relationships between staff and clients

- Designated Safeguarding Lead appointed
- All allegations investigated immediately and reported to Board
- Whistleblowing protection in place

19.5 Data Protection and Confidentiality

- Client records stored securely with access restricted to authorised clinical staff
- Digital records protected by encryption and access controls
- Information sharing only with explicit client consent or where legally mandated
- Confidentiality maintained in all external communications
- Consent forms document data handling practices

19.6 Protocol Review Schedule

Trigger	Action
Annual scheduled review	Full protocol review by Program Director and Governing Board
New Cochrane/systematic review evidence	Targeted review of affected sections
Regulatory change	Compliance review and protocol update
Serious adverse event	Emergency review, root cause analysis, protocol modification if indicated
ASAM/WHO/NIDA guideline update	Alignment review within 90 days of publication

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APPENDICES

Appendix A: Consent Forms Checklist

1. Admission Agreement
2. Informed Consent for Treatment
3. Detoxification Consent
4. Confidentiality Agreement
5. Family Confidentiality Agreement (Pathway A)
6. Belongings Receipt
7. Financial Agreement
8. Rights and Responsibilities Charter

Appendix B: Clinical Assessment Schedule

Assessment	Admission	Weekly	Fortnightly	Monthly	Discharge
C-SSRS	X	X			X
CIWA-Ar (alcohol)	X	As needed			

COWS (opioid)	X	As needed			
AUDIT	X				X
DAST-10	X				X
PHQ-9	X			X	X
GAD-7	X			X	X
URICA	X		X		X
ACE (Trauma)	X				
Vitals (BP, Pulse, Temp)	X	X			X
UDS	X			X	X
Weight / BMI	X			X	X

Appendix C: Referral Partner Directory

Partner	Services
Yaba Psychiatric Hospital	Psychiatric assessment, inpatient care, prescription management
Rights and Humanity	Legal advice and assistance
NDLEA	Regulatory liaison, national drug treatment standards
Lagos State Ministry of Health	Regulatory oversight, HIV/ART referral
Freedom Foundation Empowerment Unit	Vocational training, seed capital, employment
Freedom Foundation Educational Unit	Further education opportunities
Life Centres (This Present House)	Church placement, pastoral mentoring, fellowship
Private Psychiatric Providers	Confidential private psychiatric care
HIV/ART Services	Government or private ART facility linkage

Appendix D: ASAM Level of Care Summary

ASAM Level	Description	HOR Mapping
Level 0.5	Early Intervention	HOR Outpatient Engagement Pathway
Level 1	Outpatient Services	Not provided at HOR
Level 2.1	Intensive Outpatient	Not provided at HOR
Level 3.1	Clinically Managed Low-Intensity Residential	Not applicable
Level 3.5	**Clinically Managed High-Intensity Residential**	**HOR Primary Programme**
Level 3.7	Medically Monitored Intensive Inpatient	Exceeds HOR scope; referral
Level 4	Medically Managed Intensive Inpatient	Exceeds HOR scope; referral

Appendix E: NIDA 13 Principles Quick Reference

#	Principle	HOR Protocol Section
1	No single treatment appropriate for everyone	4, 8.1
2	Treatment needs to be readily available	4.2, 4.5
3	Effective treatment attends to multiple needs	6, 8, 10, 11
4	Treatment plan must be continually assessed and modified	8.1
5	Remaining in treatment adequate time is critical	6.1
6	Behavioral therapies are most commonly used	8.2, 8.3
7	Medications important for many patients	7, 12
8	Treatment and services must be assessed continually	8.1, 19
9	Many drug-addicted individuals have co-occurring mental disorders	5.1, 9
10	Medically assisted detoxification is first stage only	7.1

11	Treatment does not need to be voluntary to be effective	4.2, 4.5
12	Drug use during treatment must be monitored continuously	5.5
13	Test for HIV/AIDS, hepatitis, TB, and other infectious diseases	4.8

Document End

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"For freedom Christ has set us free." -- Galatians 5:1

Prepared by: Programme Development Team **Clinical Review:** Program Director (Medical Doctor) **Spiritual Review:** Chaplain / Spiritual Formation Lead **Evidence Review:** Clinical Lead (Research & Quality Assurance) **Approved by:** HOR Governing Board **Date:** April 2026 **Version:** 2.0